



ADVANCED
CALCIUM
RESEARCH
INSTITUTE

AIC THERAPY DOSING PROTOCOL 2024

- Introduction to AIC Technology
- Mechanism of Action of AIC & AIC-SG Therapy
- Sample Clinical Cases

A guideline for participating physicians only
www.aictherapy.com



ACRI DIRECTOR'S FOREWORD

Thank you for your interest in AIC Therapy.

As a director of the Advanced Calcium Research Institute, our primary objective is to shed light on the role of calcium in various diseases beyond its established association with osteoporosis. In addition, recent discoveries have provided new insights into the multifaceted involvement of calcium in numerous other medical conditions, such as autoimmune diseases, neuro-degenerative diseases, and even cancer.

With our unique positioning in collaboration with CBHI, a Canadian research organization with antiorbital calcium formulation technology, ACRI is engaged in finding physiological applications of AIC material as an effective treatment for various medical conditions.

We believe that the incorporation of AIC therapy in alternative and integrative medicine can significantly boost the success rate. For this reason, we collaborate with participating physicians to bridge the gap between discoveries to develop superior treatment options.

This dosing protocol is a work in progress. We trust that you will find this dosing protocol helpful and enlightening. We invite you to explore our website for the most up to date insights into our work.

Server Bozdogan, MD. PhD.
Molecular Geneticist
Director of ACRI



TABLE OF CONTENTS

Disclaimer	1	AIC Therapy for Cancer	47
AIC Therapy Protocol	2	AIC Dosing for Cancer	48
Expertise of ACRI-CBHI Team	3	Recommended Dosing for All Stage Cancer	49
CBHI Research Partner Organizations	4	AIC Therapy Phase One	50
Invention of New Calcium Carbonate	5	AIC Therapy Phase Two	51
Antiorbital Chemical Bonding	6	AIC Therapy Phase Three	52
Only Ionic Form is Active	7	AIC Therapy Recovery Phases	53
Absorbed by Diffusion and Osmotic Pressure	8	Multi-faceted Assaults on Cancer	54
Active vs Passive Absorption	9	AIC Therapy Exposes Cancer to Immune System	55
Physiological Effects of AIC	10	AIC-SG Therapy for Cancer	56
Functions of Calcium in the Body	11	Invention of AIC-SG Molecule	57
Most Tightly & Swiftly Regulated Molecule	12	Selective Toxicity of AIC-SG Molecule	58
Cellular Calcium Dyshomeostasis in Degenerative Disease	13	Tumor Lysis Syndrome Monitoring	59
Treatment Goals of AIC Therapy	14	Recommended AIC-SG Dosing Protocol	60
Understanding Your AIC Prescription	15	AIC-SG Treatment Guideline	61
No Cause for Hypercalcemia	16	Clinical Cases	62
Time & Water for Proper AIC Intake	17	What Do Our Patients Say	63
AIC Storage	18	Multiple Myeloma Case Study	64
About Sediments	19	De-calcification Effects of AIC	65
AIC Overdose	20	Lactic Acid Reduction Clinical Case	66
Improved Efficacy with Multiple Dosing	21	Osteoporosis Animal Trials	67
Starting with a Strong Dose	22	Sample Clinical Bone Density Increase	68
Systemic Therapy that Takes Time	23	Fracture Healing Effects of AIC	69
Healing Reactions, not Side Effects	24	Dental Cavitation & Implants	70
Types of Healing Reactions	25	Atopic Dermatitis Case	71
Minimizing Healing Pains	26	Eczema & Psoriasis Case	72
Contraindications	27	COVID-19 Clinical Study	73
Commonly Treated Conditions	28	Dementia Case with Arthritis	74
Dosing for Children	29	Dementia & Epilepsy Case	75
Dosing for CKD Patients	30	Autism Case	76
Mara-Gen IV Infusion	31	Cervical Cancer Case Study	77
Mara-Gen IV vs Oral Mara-Gen	32	Lytic Lesions in Breast Cancer	79
Mara-Gen IV Dosing Protocol	33	Ovarian Cancer AIC IV Therapy	81
Nebulizing Mara-Gen	34	Prostate Cancer with Liver Metastasis	82
AIC Dosing for Preventive Health	35	AIC-SG Therapy in Breast Cancer	83
AIC Dosing for Degenerative Disease	36	AIC-SG Therapy in Rectal Cancer	84
Neurodegeneration Dosing Protocol	37	AIC Therapy Product Line Up	86
Arthritis Dosing Protocol	38	Mara-Gen vs Mega-Gen	87
Osteoporosis Dosing Protocol	39	Mara-Gen	89
Dental Dosing Protocol	40	Mega-Gen	90
Recommended Tests for Osteoporosis	41	Carti-Gen	91
Endogenous Calcitonin Bone-building	42	Alzi-Gen	92
AIC Therapy & DEXA Measurements	43	Denti-Gen	93
Bone-loss & Degenerative Pathophysiology	44	Osso-Jr	94
Autoimmune & Infectious Disease Dosing Protocol	45	Upcoming Products	95
3 Months of Minimum Treatment	46	Appendix A: More on AIC-SG	96

DISCLAIMER

Advanced Calcium Research Institute (ACRI) thanks participating doctors, clinics, and health care practitioners for making AIC therapy available to consenting patients seeking safe alternative/integrative treatments.

Please note that ACRI hereby makes no medical claims to treat or cure any diseases. AIC-applied products are registered as dietary supplements, and the health benefits come from our body's natural response to addressing ionic calcium deficiency through AIC therapy. At the recommended dosage, AIC temporarily elevates blood ionic calcium concentration within the safe range to trigger healing responses. Simply put, AIC therapy is a nutritional therapy based on ionic calcium.

ACRI makes only one scientific claim that AIC (Antiorbital Ionic Calcium) applied products, when taken orally, deliver calcium in a free ionic state, which, being a tightly regulated signaling agent, can trigger a cascade of physiological benefits. Utilizing these physiological properties in treating diseases in any capacity is solely at the discretion of participating physicians.

This dosing protocol recommends dosing of AIC by the product line made by Pronuvia. Currently, Pronuvia is the only company with a license from CBHI to produce its medical-grade ionic calcium products. The treatment recommendations in this book are solely ACRI's recommendation to physicians and do not represent Pronuvia's position. Pronuvia makes no disease claims and does not recommend the products for any cure of diseases.

AIC therapy is practiced only by licensed physicians, clinics, and health care practitioners, and all the patient responsibility lies solely on participating practitioners and consenting patients. ACRI does not treat patients directly and encourages them to make their own healthcare decisions based on their research and in partnership with a qualified healthcare professional.

INTRODUCTION TO AIC THERAPY PROTOCOL

AIC therapy utilizes the world's first calcium-ion-delivery system called Antiorbital Bonding Molecule Calcium Carbonate (AIC- CaCO_3). When ingested with water, AIC releases calcium cations without anions, which makes ionic absorption possible, bypassing usual calcium absorption by binding with vitamin D and peptides.

AIC therapy elevates the physiologically active plasma ionic calcium level, triggering calcitonin release that up-regulates bone repair and turnover, building stronger bones while establishing intracellular healthy calcium homeostasis. Appropriating influx and efflux of calcium ions improves mitochondrial functions, reduces oxidative stress, and normalizes cellular calcium signaling, which is the key to treating chronic degenerative diseases.

It is important to note that AIC is the only ionic calcium product that delivers calcium in an ionic state and gives the aforementioned benefits. Products based on calcium chloride or calcium oxides are not absorbed in ionic form, although they contain ionic calcium. All naturally occurring calcium, whether in liquid or solid form, is absorbed in a physiologically inactive, protein-bound form.

This dosing guideline provides participating physicians with suggestions on how AIC-applied products can be utilized effectively for different types of diseases. AIC therapy is recommended as a part of a more comprehensive treatment plan. AIC therapy continues to evolve, undergoing clinical evaluations by researchers and doctors.



EXPERTISE OF ACRI-CBHI TEAM

Paul K. Lee

Chemistry, Ph.D.

Server Bozdogan

Genetics, Ph.D.

Martin Chun

Material, Ph.D.

O.S. Lee

Foods Science, Ph.D.

Edward Lee

Materials Eng., Ph.D.

James Lee

Toxicology, Ph.D.

W. S. Kim

Material, Ph.D.

Chris. Choe

Energy, Ph.D.

In today's medical world, there is no effective way to prevent age-related bone loss and consequent cellular calcification that serve as a trigger for a host of degenerative diseases. The R&D team of CBHI, a research institute based in Canada, invented antiorbital ionic calcium carbonate (AIC). AIC therapy utilizes ionic calcium as a signaling agent to trigger our body's self-healing responses to reverse cellular calcification, causing domino effects of healing processes to rebuild our health from the cellular to systemic level.

Joining hands with many healthcare practitioners and clinics around the world, the ACRI and CBHI team expands the application of AIC in treating many difficult diseases.

ACRI engages and partners with experts in the field to collaborate in new research, bringing hopes of restored health to desperate patients. ACRI is at the forefront of educating and supporting partnering physicians and clinics around the world.

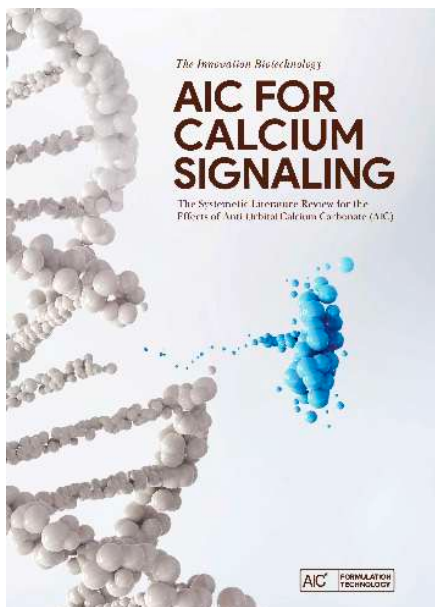
ACRI RESEARCH PARTNER ORGANIZATIONS



CBHI RESEARCH PARTNER ORGANIZATIONS



HOPE4CANCER



More technical information on AIC is found in this book. Find it in Amazon.com.

The Calcium and Bone Health Institute of Canada (CBHI Canada) is a scientific research-based institution founded in 2001 by Dr. Paul Lee that focuses on the role of calcium and its effect on our bodies.

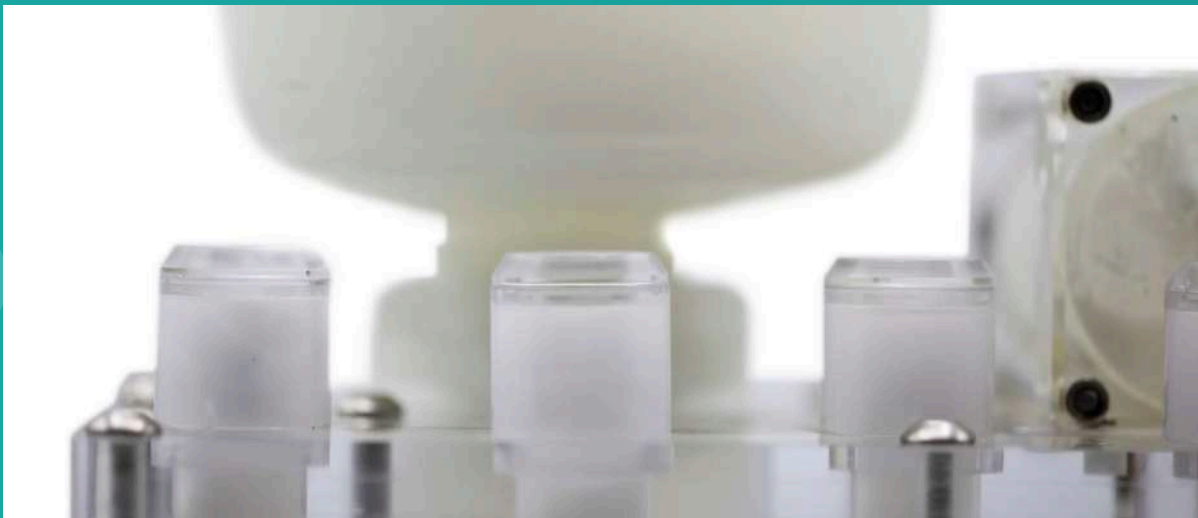
CBHI collaborates with many reputed researchers from different disciplines from SFU, UBC, UC Davis, NSERC Canada, and BC Government (BCBN). CBHI also works with many applied science faculties, such as engineering and computing science, to enhance the depth of the research.

CBHI is among the few Canadian research-based institutes that have been founded to study calcium-related chronic diseases, such as cancer, osteoporosis, Alzheimer's, and others. The partnership with ACRI (Advanced Calcium Research Institute) has helped over 100,000 people to experience the efficacy of AIC-applied ionic calcium therapy through participating physicians and clinics worldwide.

THE INVENTION OF NEW CALCIUM CARBONATE

**Antiorbital Ionic Calcium
(AIC-CaCO₃)** in a key molecule that makes AIC Therapy possible.

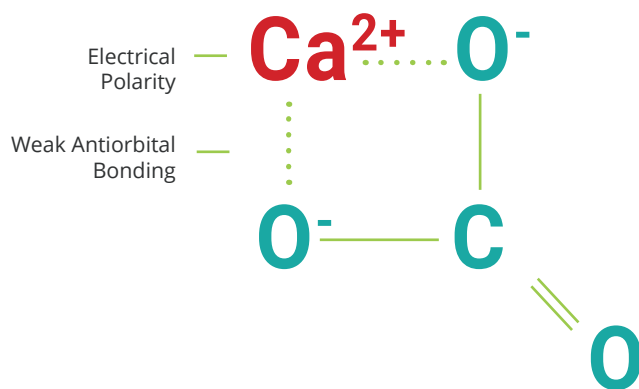
AIC is the world's first calcium-ion-delivery-system that safely and effectively achieves the threshold level of plasma ionic calcium concentration to initiate a cascade of physiological responses that awakens our body's amazing healing mechanisms!



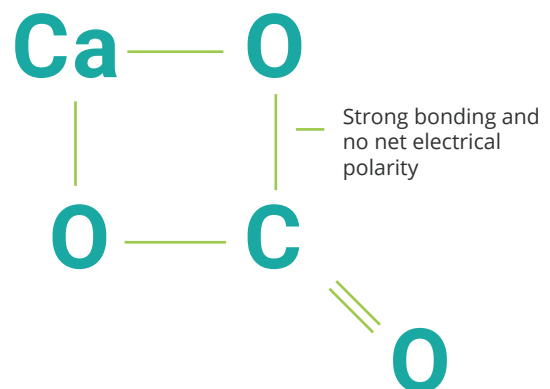
AIC UTILIZES ANTI-ORBITAL CHEMICAL BONDING

Antiorbital bonding is a very weak and unstable chemical bonding that rarely exists naturally at room temperature. They are formed when atomic orbitals combine in ways that lead to predominantly destructive interference. CBHI's ten years of R&D produced new calcium carbonate molecule with antiorbital bonding. AIC is 200x more soluble and exhibits different chemical and physical properties compared to calcium carbonate. Taken orally, its unique ionic absorption triggers a cascade of hormonal responses, including osteoblast to build healthy bone.

AIC CALCIUM CARBONATE



REGULAR CALCIUM CARBONATE

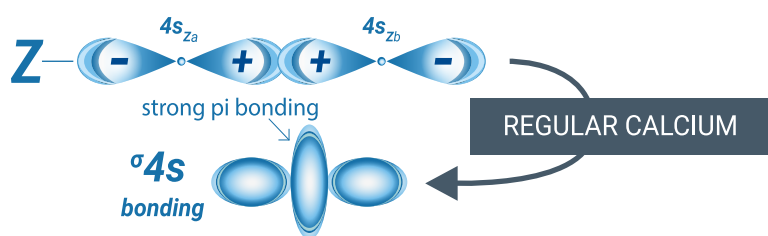
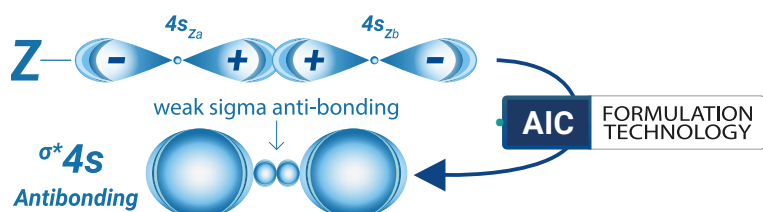


UNIQUE PHYSICAL PROPERTY OF AIC

AIC molecules have active hydrogen bonds due to +2 charges and fly out together with water molecules to crystallize with CO_2 in the air.

ONLY IONIC FORM IS ACTIVE

Ionic calcium (Ca^{2+}) is the only physiologically active form that is used as signaling molecules and therefore, its concentration is tightly and speedily regulated by our body's homeostatic system. All dietary calcium intake is done in protein-bound inactive form as not to disrupt the delicate homeostasis.



AIC is 200x more soluble in water and 3x more reactive in chemical reactions due to its weak Antiorbital bonding. Calcium ions are easily released due to weak bonding.

**CALCIUM
CONTENT
IN NORMAL
BLOOD**

50%

Ionic
Calcium

45%

Protein
Calcium

5%

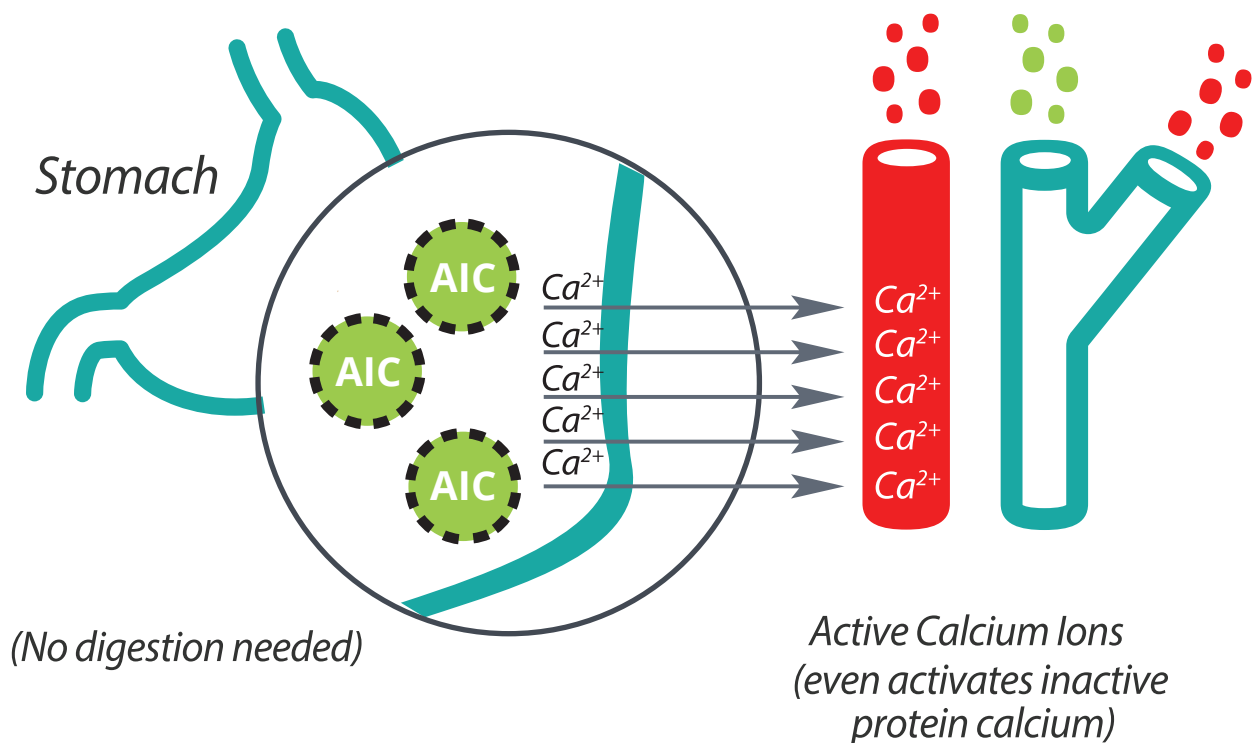
Inorganic
Calcium

Our blood serum has 50% of calcium in ionic form, and increasing it by 1-2% by AIC triggers the bone formation process (osteoblast) and restores calcium homeostasis.

ABSORBED BY DIFFUSION & OSMOTIC PRESSURE

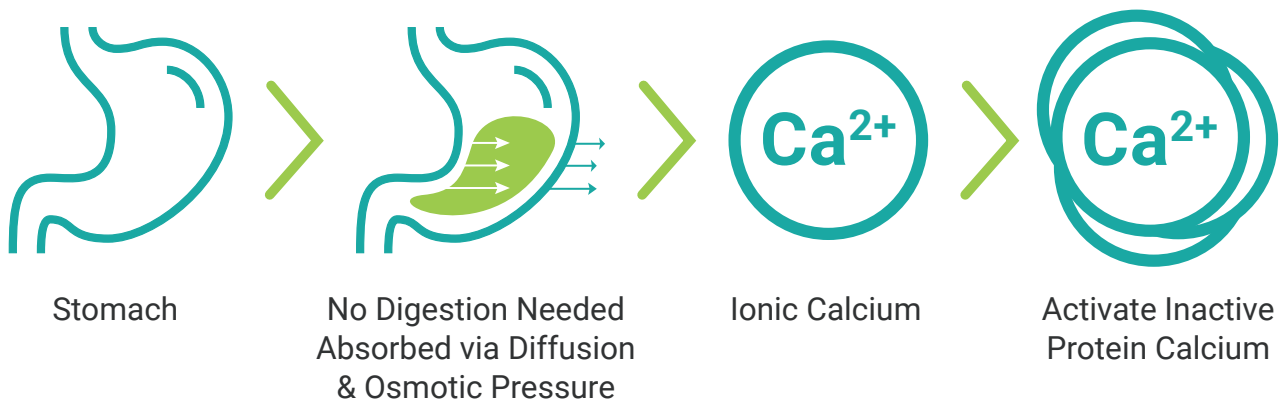
- No digestion required.
- Direct absorption through stomach wall.
- No vitamin D3 and peptides required for absorption

Passive Transport (Direct absorption with water)

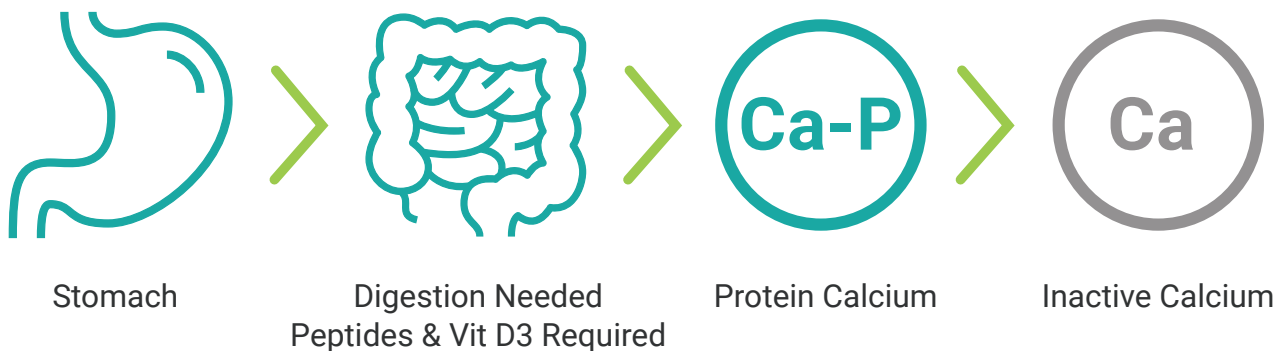


ACTIVE vs. PASSIVE ABSORPTION

PASSIVE TRANSPORT (ONLY AIC)



ACTIVE TRANSPORT (CALCIUM FROM DIET & SUPPLEMENTS)



ANY OTHER IONIC CALCIUM AVAILABLE ?

All calcium has ionic calcium when it is digested; however, none of that is going to be absorbed in ionic form. Even calcium chloride and calcium oxide that dissolve well in water will not be absorbed in ionic form. Only AIC with anti-orbital bonding will bypass our body's natural calcium absorption pathway as free calcium ions: Ca^{2+}

Active Transport applies to calcium from both diet and calcium supplements such as calcium carbonate, calcium citrate, calcium lactate, calcium gluconate, coral calcium, etc. Only AIC is immediately utilized as active calcium to aid with bone and dental health, regulate hormones, maintain ideal body pH, and restore cellular calcium homeostasis.

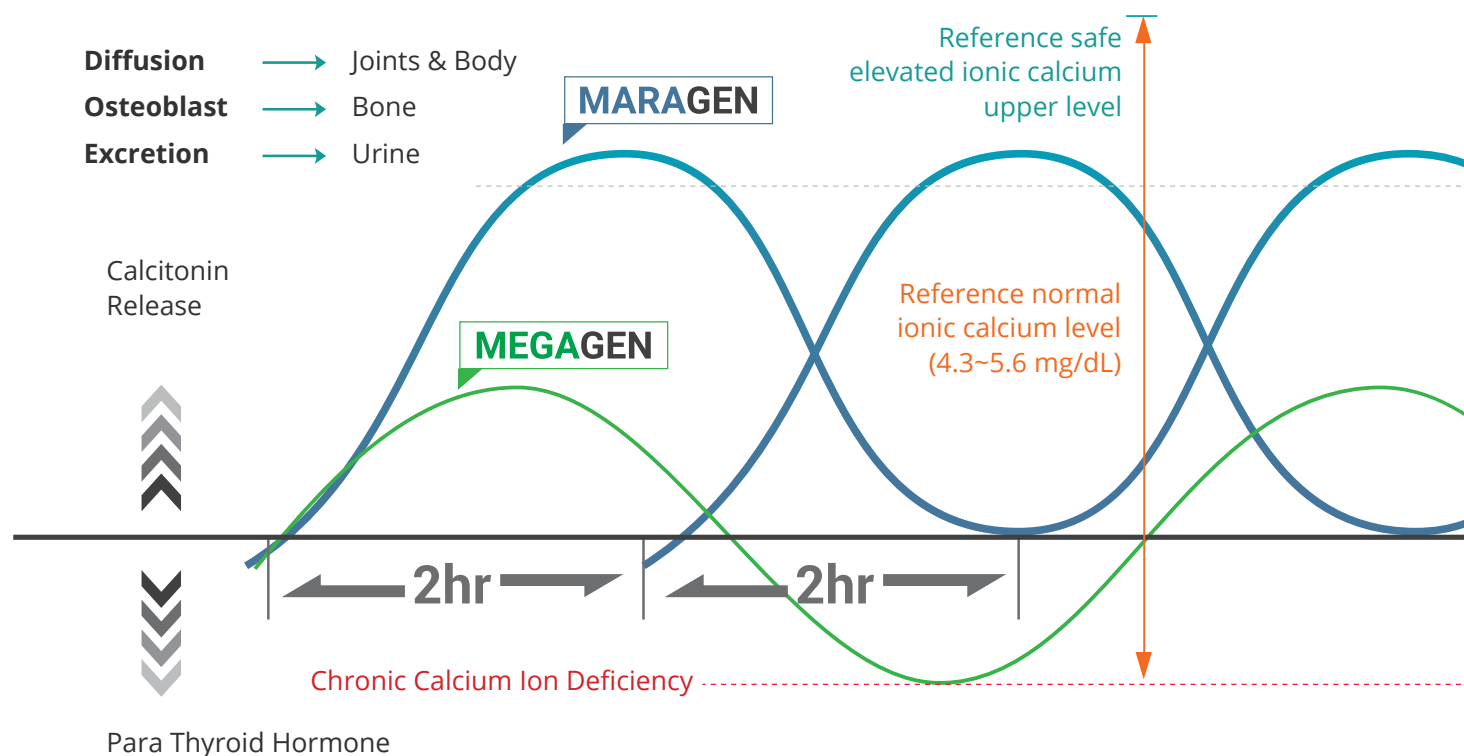
PHYSIOLOGICAL EFFECTS OF AIC

After intake, AIC's effect lasts about four hours in our body, slightly raising the serum ionic calcium concentration level safely to trigger various physiological functions as our body brings down the level to the average physiological level.

Elevated ionic calcium triggers the release of TSH which in turn triggers the release of calcitonin. **Calcitonin** inhibits osteoclast function but increases osteoblast function indirectly to up-regulate bone turnover rate which builds and repairs bone. Osteoblast cells in turn secrete **osteocalcin**, an important hormone with metabolic benefits such as beta-cell production and insulin sensitivity. Osteocalcin also improves memory.

While serum ionic calcium level is elevated, kidneys excrete excess ionic calcium by urination. The urge to urinate within an hour of taking AIC is a both healthy and normal reaction, an indication that AIC is working. However, when the patient is in ionic calcium deficiency, the urge to urinate is much less.

As AIC initiates the bone-building process, inactive protein-calcium is also activated, releasing both ionic calcium and protein for use. Using Mara-Gen multiple doses a day can put our body in constant bone building mode and trigger body-wide de-calcification effects, especially in cellular level.



FUNCTIONS OF CALCIUM IN THE BODY

- Muscle and Heart Contraction
- Bone and Teeth / Blood Clotting
- Stem Cell Regulation
- Neurotransmitter / Second Messenger
- Hormone secretion / Enzyme Activation
- Immune system / Cell Membrane Stability
- Cell Cycle and Repair / Autophagy / Apoptosis
- Transcription / Mitochondria Functions

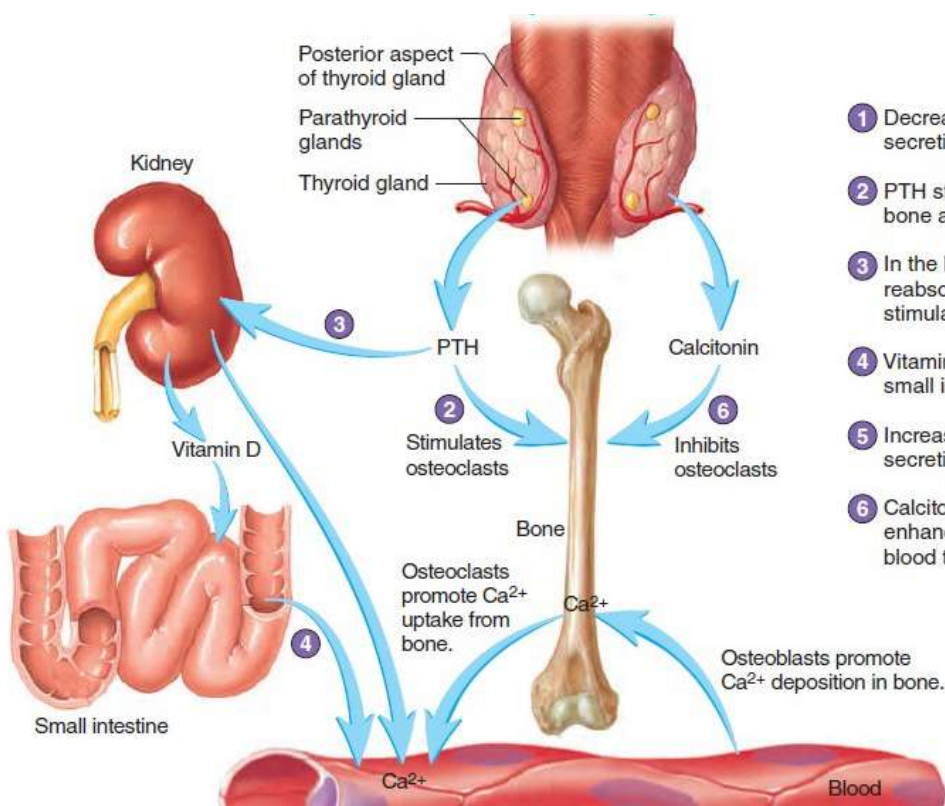
Because of calcium's role in cellular communications, precise regulation of its concentration is required for proper physiological functions of all systems of our body. Once the balance is tipped, the disease is inevitable.



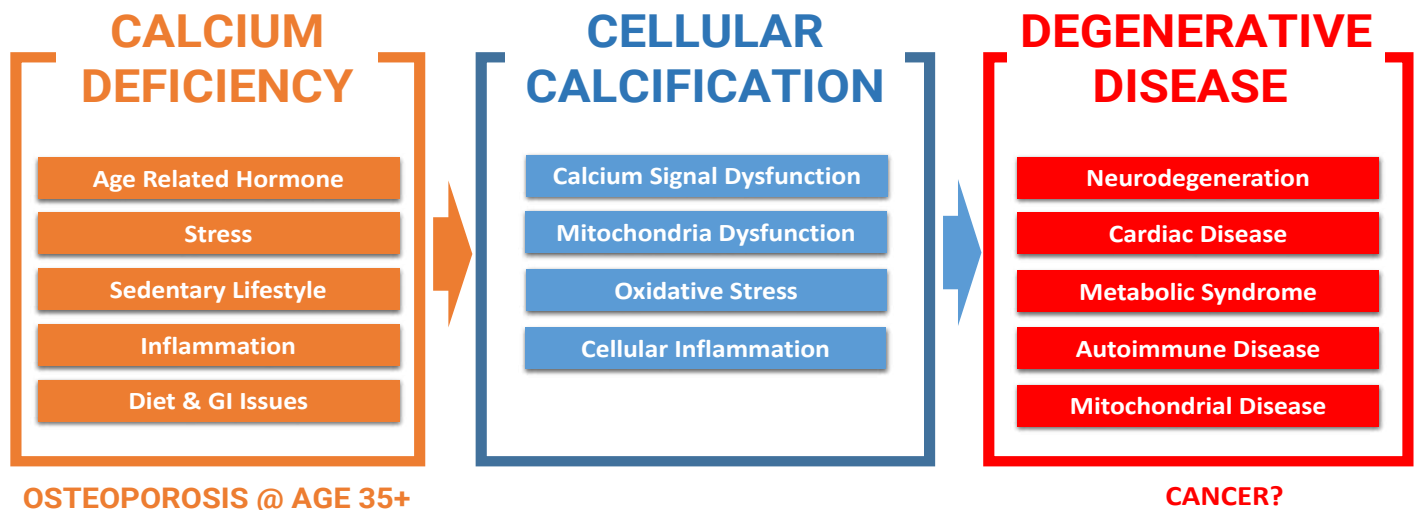
MOST TIGHTLY & SWIFTLY REGULATED MOLECULE

No ion or molecule in our body is as tightly and speedily regulated as ionic calcium. When serum ionic calcium slightly rises, immediately calcitonin release is triggered to build bones and bring its level down to homeostatic level. When ionic calcium level dips slightly, PTH is release to resorb bone. Active regulation is required to maintain precise concentration level of this signaling molecule in extracellular and intracellular level.

Important cellular functions such as transcription, enzyme activation, and cell repair are communicated via complex calcium signaling. Unfortunately, with aging related bone-loss and pathogenic toxic pathways, calcium signals are often compromised, resulting in functional breakdown. This is the main driving force for both aging and degenerative diseases.



CELLULAR CALCIUM DYSHOMEOSTASIS IN DEGENERATIVE DISEASE



Many things can lead to ionic calcium deficiency: improper diet, sedentary lifestyle, age related hormonal imbalance, stress, inflammation, etc. Our body resorbs more bones trying to compensate for the deficiency, which unfortunately leads to more cellular calcification. Cellular calcium dys-homeostasis is one of the main cause of degenerative diseases. The goal of AIC therapy is triggering strong bone building that could navigate displaced calcium back to our bones, especially from cellular spaces. De-calcification as many healing properties.

CELLULAR DE-CALCIFICATION

- Reduces Oxidative Stress
- Corrects Calcium Signaling
- Increases Mitochondrial Function
- Reduces Cellular Inflammation

A man and a woman are jogging through a lush, green park. The woman is in the foreground, wearing a light blue long-sleeved shirt and pink leggings, smiling. The man is slightly behind her, wearing a light blue t-shirt and white shorts. They are surrounded by tall trees with dense foliage. A teal banner is overlaid on the bottom half of the image, containing the title.

TREATMENT GOALS OF AIC THERAPY

Ionic calcium is critical in cellular communication, functions as signaling molecule, neurotransmitter, and secondary messenger, and therefore is tightly regulated by our body from brain to heart and to immune system.

However, with aging, poor nutrition, unhealthy lifestyle, and accumulation of toxins from food and the environment, the delicate balance of calcium homeostasis starts to tip over, causing both cellular and systemic communications breakdowns, which in turn triggers the onset of a host of degenerative diseases.

Being the world's first true ionic-calcium supplement, AIC triggers our body's natural calcium-sensitive responses that help to restore the delicate balance of calcium homeostasis, which normalizes cellular communication, restores mitochondrial functions, and reduces oxidative stress. Healthy cells lead to healthy tissues, organs, and systems, and this cells-to-systems overhaul by AIC therapy is the way how the therapy helps to reverse degenerative diseases and improve quality of life.

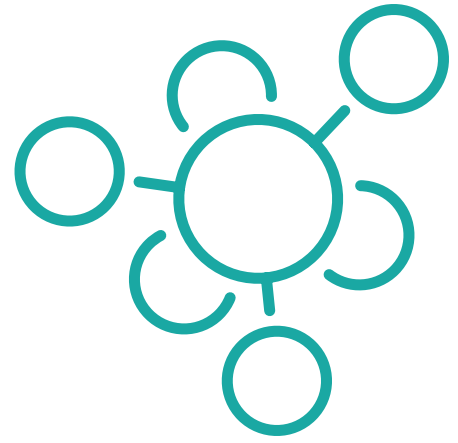


UNDERSTANDING YOUR AIC PRESCRIPTION

Our body's calcium homeostatic system safely and effectively regulates fluctuations of serum plasma ionic calcium concentrations, and AIC therapy provides the dosage intake well within the healthy range. AIC has no known side effects or contraindications with known drugs, and for many years doctors and health practitioners around the world employed AIC as an adjunct or adjuvant treatment or even as a stand-alone primary treatment for their patients with much success.

Because of AIC's usage in treating many severe diseases alongside both conventional and integrative treatments, AIC therapy is provided only through participating doctors with proper knowledge and training in AIC. Prescribing effecting dosage, managing patient's expectations, and monitoring progress are required for AIC therapy to maximize the patient benefits and to arrive at the desired health outcome without complications.

For these reasons, AIC-applied products are not sold over the counter or online, unlike other supplements, but are dispensed only under the care of participating health practitioners. AIC is to be treated as therapy with strict protocols and should only be prescribed as such.



NO CAUSE FOR HYPERCALCEMIA

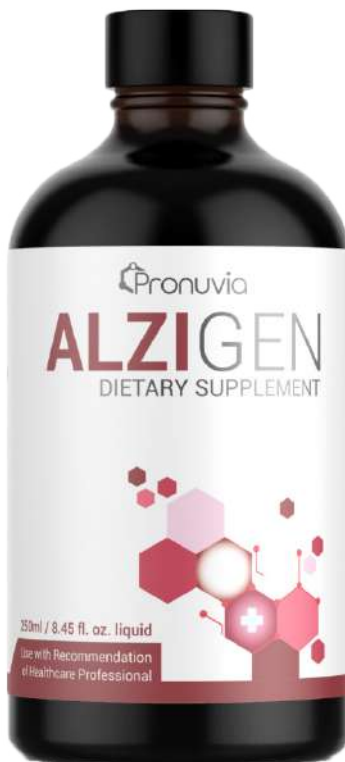
HYPOCALCEMIA

NORMAL 4.3-5.6 mg/dL

MILD HYPERCALCEMIA

8mg/dL

Amount of Ca^{2+} added by AIC per dose is within normal/safe range, causing about 1-2% change in the concentration.



- AIC adds only 5mg-8mg of Ca^{2+} to 5 L of blood (average adult), which increases the concentration of ionic calcium only by 0.15mg/dL max, working well within the safe range.
- Hypercalcemia is classified with plasma ionized calcium levels above 5.6 mg/dL (Mild Hypercalcemia : Ca^{2+} @ 5.6 ~ 8mg/dL)
- The total amount of Ca^{2+} in an average adult is about 250mg. AIC only adds about 5-8 mg, which is about 2-3% increase, well within the safe range.
- Hyperparathyroidism causes hypercalcemia. Calcitonin secretion triggered by ionic calcium results in increased bone turnover rate, which reduces hypercalcemia by carrying excess calcium back to our bones and inhibiting PTH function.
- Carrying excess phosphorus to bones together with calcium during the bone-building process, AIC lowers our body's calcification factor and prevents or reverses calcification.



TIME & WATER FOR PROPER AIC INTAKE

AIC has to be mixed with room temperature or cold water at 1:100 ratio and taken on an empty stomach at least 2 hours after and 30 minutes before any food intake. More than 95% of AIC will be absorbed within 30 minutes through diffusion and osmotic pressure.

Most kinds of water can be used with AIC. Alkaline water, hydrogen water, filtered water, reverse-osmosis water, and non-carbonated mineral water work well with AIC. Any additives such as other supplements or food substance that may interact with AIC should be avoided as AIC will lose efficacy. Taking AIC without mixing it with water will render it useless as ionic calcium therapy and ample amount of water is needed to expand stomach and increase osmotic pressure. Also, the water temperature must not exceed above 55 °C.

The suggested time slots for intake are soon after waking up, 30 min before lunch, mid afternoon between lunch and dinner, 30 min before dinner, and one or two hours before bed. Drinking much water may be difficult for some patients; however, drinking 2 L of water is recommended by many health experts.



AIC STORAGE

Storage: 0 - 55°C (32 - 130°F) away from direct sunlight.

Un-opened Bottle Expiration : 10+ years

Opened Bottle Expiration: 6 month from opening

Consumption Time Period: 10 minutes from mixing

AIC comes in colored glass bottles for a more extended preservation period. Unless the product is not exposed to direct sunlight and stored below 55°C (131°F) for AIC and 80°C (176°F) for AIC-SG, unopened bottles of AIC will last more than ten years without losing potency. Once the bottle is opened, the product is to be consumed within six months without experiencing the loss of therapeutic potency.

As bottles are opened and closed, the air introduced can react slightly with AIC and cause minute precipitation of calcium carbonate, making the solution more cloudy, but it won't affect efficacy much. The bottle cap should be tightly closed immediately after each dosing to minimize the exposure to air.

Once mixed with water, the solution should preferably be consumed within 10 minutes and not exceeding 30 minutes. If the solution is premixed in a tightly enclosed plastic container for later consumption, please make sure that the solution is not exposed to direct sunlight and consumed within 6 hours from the time of the mix. The container should not be too big to contain much air. However, for the best results, it is highly recommended that AIC is mixed just prior to consumption.

No pathogens can survive in AIC solution due to the presence of a high concentration of ionic calcium. AIC is sterile in the bottle.





ABOUT SEDIMENTS

AIC particles typically exhibit sub-nanometer dimensions, rendering them visually soluble. Nevertheless, a marginal increase in the concentration of AIC can prompt particle agglomeration, propelled by electrostatic and magnetic forces within the local environment. Beyond the 1 nm threshold, a transition to a colloidal phase is observed, characterized by the formation of low-density aggregates. Remarkably, these colloidal entities exhibit consistent physical properties upon dilution in aqueous or saline solutions, underscoring the persistence of the agglomeration phenomenon intrinsic to AIC and AIC-SG.

During the AIC-SG manufacturing process, AIC engages in a reaction with glucose, necessitating strict control to ensure the equivalent weight of AIC calcium equals or surpasses that of glucose. A surplus of AIC is conventionally employed to minimize unreacted glucose in the final stages, strategically preserving excess AIC in the product as a safeguard against material degradation. This surplus is particularly vital due to the antibacterial properties of AIC, which curtail bacterial proliferation. Additionally, the surplus of AIC bestows antimicrobial attributes, potentially conferring the product with an indefinitely extended shelf life. This holistic approach in AIC manufacturing aligns meticulously with exacting standards for quality, stability, and therapeutic efficacy in advanced biomedical applications.

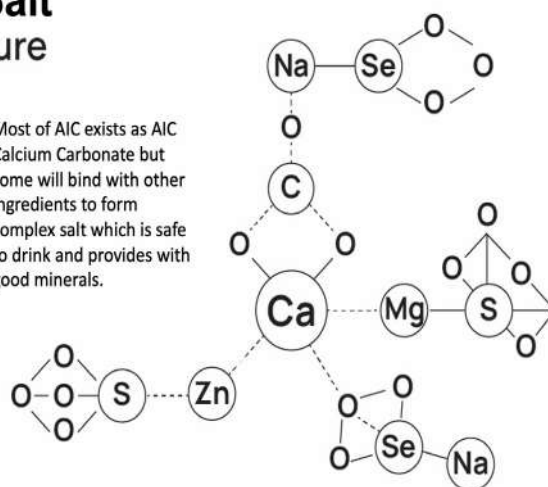
The incidence of sedimentation is common, and even in its presence or absence, the consistency of AIC Calcium content remains intact. A pragmatic method for assessing product integrity involves the application of AIC solution to litmus test paper. The conversion of yellow litmus paper to purple, indicative of alkalinity upon interaction with the solution, serves as a reliable proxy for product quality.

The scarcity of precipitation is ascribed to the manifestation of a colloidal phase, characterized by complex or double salts involving selenium, magnesium, zinc, and calcium. The propensity for colloidal precipitation diminishes when the vial material contains substantial quantities of cationic metal ions. This observation accentuates the robust stability and composition of AIC, ensuring the sustained presence of AIC's efficacy in either presence or absence of sedimentation. This analytical methodology adheres rigorously to stringent quality assessment criteria, affirming the reliability of AIC products in preserving their chemical attributes.

AIC Complex Salt Molecular Structure



Most of AIC exists as AIC Calcium Carbonate but some will bind with other ingredients to form complex salt which is safe to drink and provides with good minerals.



A photograph of a middle-aged man with grey hair and a beard, wearing a white t-shirt, drinking water from a clear plastic bottle. He is outdoors, with a blurred background of green trees and foliage. The lighting is bright, suggesting a sunny day. The image is used as a background for the text overlay.

AIC OVERDOSE?

With AIC therapy, generally, there is no overdosing risks. If a person drinks an entire bottle of AIC without mixing with the proper amount of water, AIC is just absorbed as regular calcium carbonate (180 mg of calcium as calcium carbonate in a 120ml size bottle), and none is absorbed in ionic form.

If AIC is properly mixed in prescribed 500 ml of water, the amount of water will naturally limit overdosing as an average person can only drink so much water at a time.

If more AIC is added to the prescribed 500ml of water, its absorption rate will decrease, which also limits the absorption of ionic calcium. Our body also has very effective system that regulates serum ionic calcium concentration. Therefore, excessive concentration of ionic calcium circulating in the blood, if happened, is excreted in the urine.

The only concern of overdosing is with the selenium amount as each AIC dose provides 80% (40mcg) of the required daily value. Over 300 mcg of selenium for an extended period of time may cause health issues. Taking Mara-Gen 6 doses a day remains well within the safe range of the selenium intake.

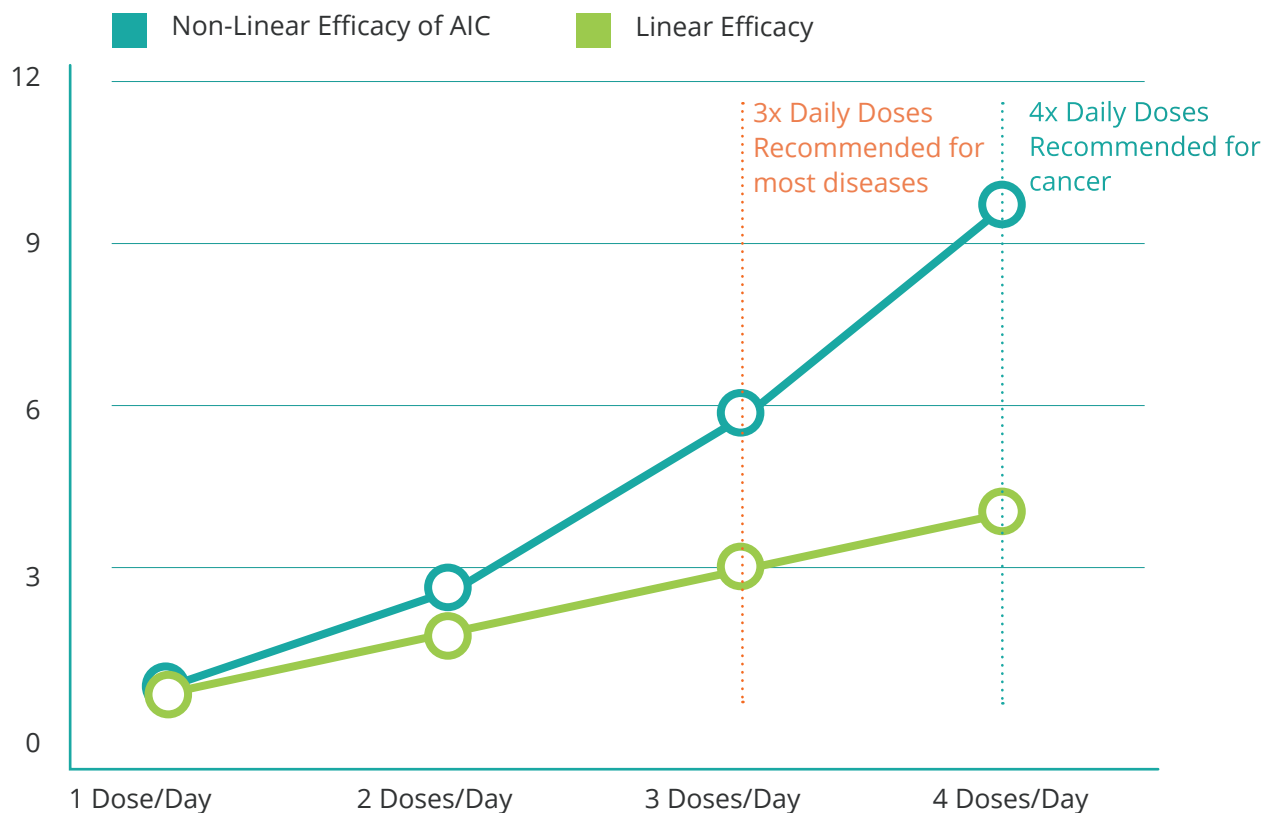
IMPROVED EFFICACY WITH MULTIPLE DOSING

Efficacy of AIC therapy increases non-linearly with the number of doses each day. For instance, taking AIC twice a day has more than double the efficacy of taking AIC only once a day in our clinical experiences. It is comparable to two people carrying a table instead of just one. Two can handle the job much easier.

Therefore, by taking AIC many times a day, one can experience multiplied AIC efficacy compared to taking just one dose a day and able to curb the disease momentum and shorten the therapy duration, making AIC therapy more cost-effective.

There are no side effects of taking AIC multiple doses a day as our body is able to manage small fluctuations of plasma ionic calcium levels.

By taking multiple doses a day, AIC therapy has a better chance of fighting the disease momentum and speeding up the recovery time. All treatments, therefore, should start at three doses a day, except cancer which requires minimum four a day.



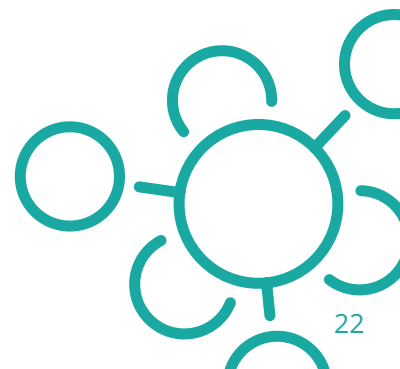


STARTING WITH A STRONG DOSE

A person does not go from being healthy to diseases state overnight, unless it involves injury or infection. Therefore, when a patient is diagnosed with a degenerative disease, the progression happened for a period of time before the manifestation of evident symptoms. Without intervention, the diseased condition inevitably deteriorates with rising momentum. To stop and reverse this disease momentum, it is always better to push back strong from the onset of the treatment.

Patients with diseases, therefore, should start with Mara-Gen (strongest AIC product) at three doses a day for all conditions except cancer for which we recommend a minimum of 4 doses a day. We recommend going strong in the beginning and titrate down to maintenance dosing once health is restored.

Most patients have multiple health issues that are related to ionic calcium dysfunction. From cardiovascular disease to metabolic syndrome to autoimmune disease, ionic calcium from AIC must fight multiple fronts to help patients to recover. Therefore, the need for AIC is greatest on the day one of the treatment.

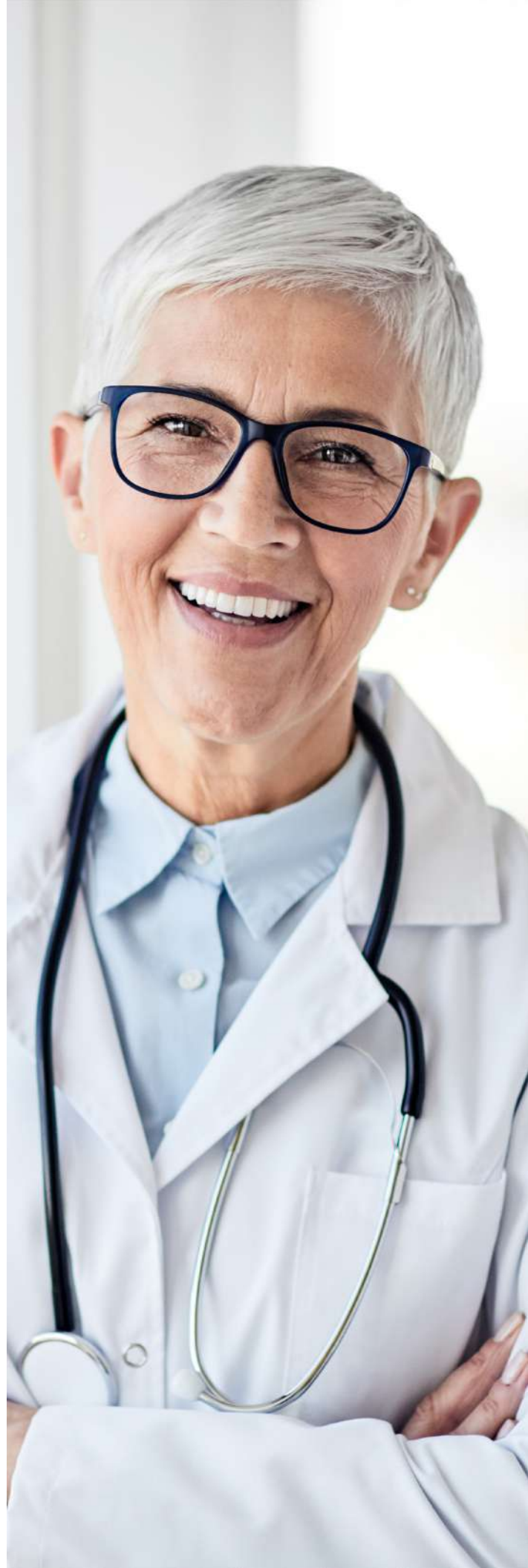


SYSTEMIC THERAPY THAT TAKES TIME

AIC therapy does not target symptoms alleviation first. Rather, AIC is a systemic therapy that focuses on treating the root causes of the disease from the cellular level up. Therefore, it generally takes longer to clinically experience the positive effects of AIC therapy compared to symptom-targeted treatments. However, once the disease momentum is reversed, the recovery often accelerates with results that are far more outreaching, including reversal of the symptoms.

Restoration of calcium homeostasis at cellular level restores mitochondrial functions, reduces oxidative stress, and repairs dysfunctional cellular communication. As cellular health is enhanced, AIC therapy addresses debilitating systemic issues and switch on our body's strong healing abilities to eradicate the disease; however, both time and patience are required from patients. From healthy cells to healthy tissues to healthy organs to healthy systems, AIC's work needs time.

The anticipated rate of success of AIC therapy depends on several variables, including the patient's medical history, current physical condition, lifestyle, compliance, age, ability to heal, emotional state, etc. Therefore, the care provided by physicians addressing the overall condition of the patient will deliver more rewarding results from AIC therapy protocols.



A photograph of a middle-aged man with grey hair and sunglasses, wearing a blue shirt, drinking water from a clear plastic bottle. He is outdoors, with a blurred background of green trees and a bright, hazy sky. A green rectangular banner is overlaid on the bottom half of the image, containing the title text in white.

HEALING REACTIONS, NOT SIDE EFFECTS

When a patient is in a diseased state, the body experiences physiological imbalance on many fronts. Because AIC works at cellular and systemic level by restoring proper cellular communications and rebuilding cellular functions, the body in response tries to regain its balance again. Therefore, some of the reactions to the therapy may include temporary discomfort, herxheimer, and amplified symptoms, especially in treating inflammatory conditions.

Healing reactions should not be confused with side effects, which is caused by unwanted adverse effects of medications and is considered harmful. AIC has no known side effects nor contraindications with other supplements or drugs. Any drugs that interact with ionic calcium cannot be approved as a safe drug by FDA in the first place, and AIC modulates serum ionic calcium level only by 2% per dose, falling well within the safe range.

Healing reactions of AIC therapy is also a positive sign that AIC is indeed working, and therefore, many patients endure through the transitional period. However, in rare cases where the reaction is overwhelming for patients to bear, reducing the number of daily doses is recommended until the full protocol can resume again; slowly titrating to the recommended daily dosing. Healing reactions occur at vary degrees (mostly minor) to about 30% of the patients, and they last from days to weeks.



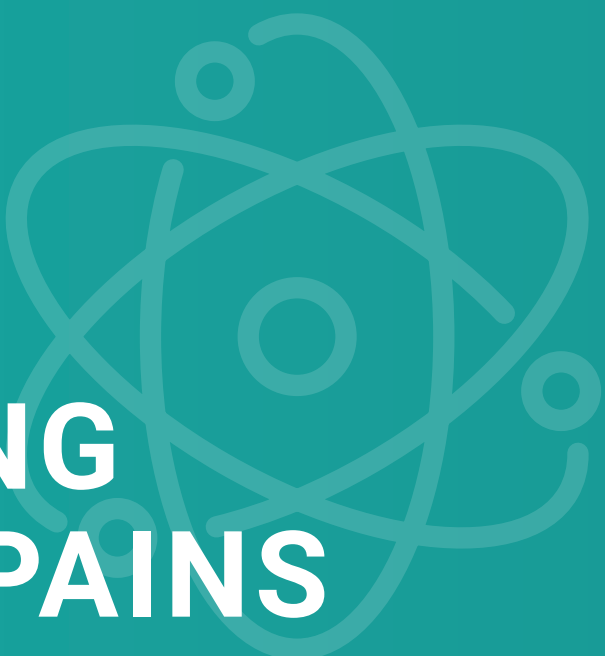
TYPES OF HEALING REACTIONS

Because AIC therapy works at most basic and cellular levels to restore the balance destroyed by disease momentum, the healing process sometimes involves unwanted pains and discomforts. **Some temporary and common healing reactions include minor headaches, upset stomach, constipation, diarrhea, light fever, aches in joints and muscles, skin rash (due to detox), joint inflammations, fatigue, sleepiness, etc.**

Patients respond differently based on their health conditions. For example, patients with heavy metal toxicity often experience skin rash as cells try to expel toxic metals such as lead and mercury. Patients with injuries may experience pains around old injuries as body tries to repair. For other patients, it is the effects of re-balancing of hormones. Still, others are the effects of restoring gut microbiome and gastrointestinal tract health.

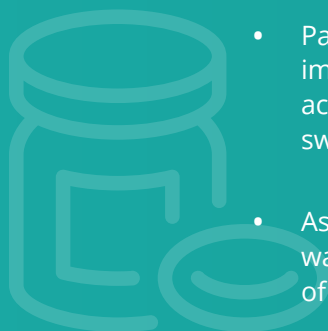
Most patients do not experience any healing reactions, while others do experience it at varying degrees. Patients with advanced disease conditions with low bone density and heavy calcifications tend to experience more healing crisis. For most people, these reactions are light and short-lived, but for a small percentage of patients with substantial calcium problems (calcification) and toxins build-up, symptoms may last anywhere from weeks to months.

When healing reactions are too overwhelming, patients are advised to reduce the dosage to the level where the healing reactions are manageable or to stop AIC therapy altogether before restarting again at a lower dosage and slowly increasing to the recommended dosage while building tolerance.



MINIMIZING HEALING PAINS

- Proper nutrition is crucial for repairing health. As AIC initiates a cascade of responses in our body to bring deep healing, proper nutrient, hydration, and detoxification is recommended to alleviate or to remove healing pains as well as to accelerate healing. When our cellular metabolism is improved, micro-nutrients are gobbled up at an accelerated pace and therefore, need to be monitored and replaced as needed. **Micronutrient tests are recommended within the first month of taking AIC to supplement properly.** Serum calcium levels may drop initially for some patients as calcium is better utilized and should be of no alarm.
- Magnesium oil can be applied to joint inflammation to ease the healing pain if it happens. Oral magnesium supplements also help. (200-500 mg)
- Vitamin D is not needed for AIC absorption; however, vitamin D is an important hormone that is needed for the proper immune functions. Vitamin D level around 70 ng/ml is recommended while taking AIC and preferably more for cancer patients.
- Patients who do not experience good bone density increases undergoing AIC therapy may try taking iodine supplements. (5-10 mg a day according to the patient's deficiency level)
- Patients who have been using steroids may experience inflammation as AIC therapy strengthens and corrects the activity of the immune system.
- Patients with low body temperature may experience body aches as the immune system is strengthened and as the body tries to detox, bringing acidic pH to the ideal 7.4. Body temperature rise may accompany much sweating for some patients.
- As our GI tract gets healthier, detoxing may cause diarrhea, or more water absorption may cause constipation. Magnesium supplementation of 500mg a day helps with relieving AIC-induced constipation.



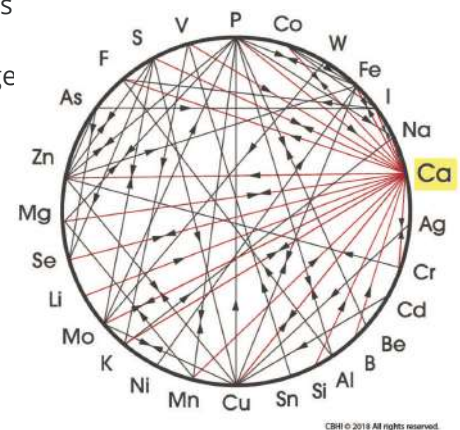
CONTRAINDICATIONS

AIC therapy provides what is already present plenty in our body: ionic calcium. An average adult body has about 250mg of ionic calcium doing its work everywhere. Adding a tiny amount of ionic calcium to trigger and induce natural responses, therefore, has no side effects and contraindications with other supplements or drugs.

However, because osteoporosis drugs interfere with the natural bone turnover processes which AIC utilizes in building bones naturally, AIC works only when osteoporosis drugs (antiresorptive drugs such as bisphosphonates and denosumab) are stopped and out of our system. Please wait 3-6 months after stopping the drugs before AIC based bone-building starts.

If a patient is taking antibiotics, please space it with the intake of AIC for 2-3 hours

If a patient is taking medicines for chronic conditions, please monitor drug dosage as patients' condition improves (diabetes, hypertension, cardiovascular disease, etc.).



MINERAL INTERACTION:

As calcium homeostasis is restored by AIC therapy, other mineral functions in our body improve because ionic calcium plays a leading role. As naturally existing in our body, ionic calcium levels are precisely regulated and do not adversely react with other supplements and drugs.





COMMONLY TREATED CONDITIONS

- Autoimmune disease (Lupus, Vitiligo, Hashimoto's, Crohn's, Celiac disease, eczema, MS, rheumatoid, etc)
- Lyme disease, HIV, MERs, shingles, Post polio syndrome, and other viral infections such as COVID
- Parkinson's, ALS, Alzheimer's and other Neurodegenerative Diseases
- Arthritis, Gout, CPPD, Inflammations
- Mitochondrial Disease
- Cancer (carcinoma, sarcoma, lymphoma, leukemia, multiple myeloma, bone metastasis)
- Osteoporosis, Bone Necrosis, Fracture Healing, Thrombosis, Hemolytic Anemia
- Arrhythmia, Heart palpitation, Mitral valve prolapse
- Hyper & hypothyroidism, Hormonal Imbalance
- Diabetes, Metabolic Syndrome
- Glaucoma, Cataract, Intermittent Exotropia, Retinal Vein Occlusion
- Autism Spectrum Disorder, ADHD, Epilepsy
- Asthma, COPD
- Menier's Disease, Vertigo, Aura Migraine Disease, Tinnitus, Anemia
- Chromosome 8 syndrome
- Chronic Kidney Disease
- Gum disease, Loose teeth
- Calcium related Infertility, Dysmenorrhea, Irregular menstruation
- Calcification (joints and tissues), kidney stones, gallbladder stones, Fibrosis, Calcific tendonitis
- And many more. There are more than 150 calcium related diseases that causes secondary autoimmune and inflammatory responses.



DOSING FOR CHILDREN

All AIC products are formulated to 60kg (132lb) bodyweight. For children, the dosage and the water amount should be adjusted to the correct bodyweight, keeping AIC to water ratio to 1:100. For example, a child with 30kg of weight should take 2.5mg of Mara-Gen in 250ml of water as one dose instead of 5ml in 500ml as in adult dosage. Teens with adult weight (50 to 70kg/110-154lb) should go with adult dosage.

For treatment purposes, Mara-Gen is recommended. However, for healthy kids to maximize growth potential and to help with mental sharpness, Mega-Gen is recommended. Mara-Gen is safe even for babies and is to be administered through a baby feeding bottle. (ex. 0.5ml AIC in 50ml of water for 6kg baby)

AIC is helpful in improving conditions such as ADHD and ASD, as many research implicated the importance of ionic calcium homeostasis in growing brain.





DOSING FOR CKD PATIENTS

AIC Absorption & Water Amount

Oral Intake is limited by the patient's ability to drink 500 ml of water as the amount of water determines the extent of stomach expansion as AIC is absorbed by diffusion and osmotic pressure.

AIC products are formulated to deliver the ideal amount of AIC for each product formulation at 5 ml of solution mixed into 500 ml of water. (at 1:100 ratio) This means that the higher AIC solution to water ratio by putting more AIC into 500ml water will reduce the absorption rate, and any unabsorbed AIC will be absorbed as protein-bound calcium in the duodenum by binding to vitamin D and not absorbed as ionic calcium.

For patients with limited fluid intake such as Chronic Kidney Disease (CKD) patients (above GFR 10), 2-3 doses/day of Mara-Gen is recommended but with a reduced amount of water (350 ml to 400 ml) per dose, not exceeding the daily fluid intake limit set by the doctor. Absorption is compromised by 20-30% due to a lower AIC to water ratio (less than 1:100), depending on the stomach size and condition. However, it is better to take more doses of AIC than less.

AIC therapy does not put burdens on kidneys. However, for more severe patients who cannot drink much water at all, Mara-Gen IV treatment is recommended since it gives more potent AIC therapy with much less fluid intake. (Please refer to Mara-Gen IV therapy)



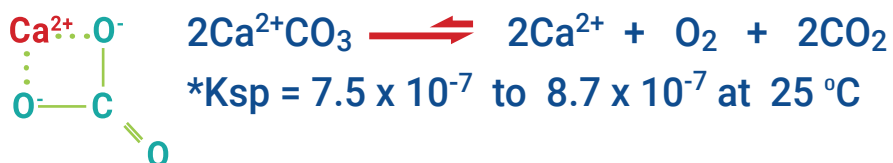
MARA-GEN IV INFUSION

For patients with fluid intake limitations such as with CKD, Mara-Gen comes in IV form, which can be infused in an intravenous drip. The difference between oral Mara-Gen and Mara-Gen IV is that there are no other ingredients such as zinc, magnesium, and selenium in Mara-Gen IV. It is a transparent liquid with only AIC.

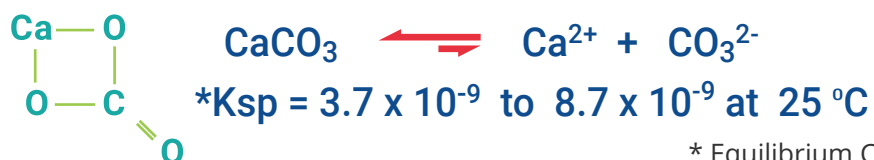
Because of AIC's weak antiorbital bonding, AIC entering the blood vessels disintegrates readily into ionic calcium, oxygen, and carbon dioxide due to -60 mV of weak electricity flowing in our body. Mara-Gen IV comes in the 120ml glass bottle container and not packaged in IV vials or ampules.

During storage, Mara-Gen IV may build precipitation (calcium carbonate), which shows as suspended solids when the bottle is shaken. To draw out a dose of AIC, a syringe with a syringe filter (0.22 μm) attached should be used to filter out precipitates that may irritate veins. There is no issue with contaminations since AIC is sterile, just like alcohol, and no pathogens can survive in it. Mara-Gen IV should be administered only in countries where intravenous treatment is legal and only by licensed practitioners.

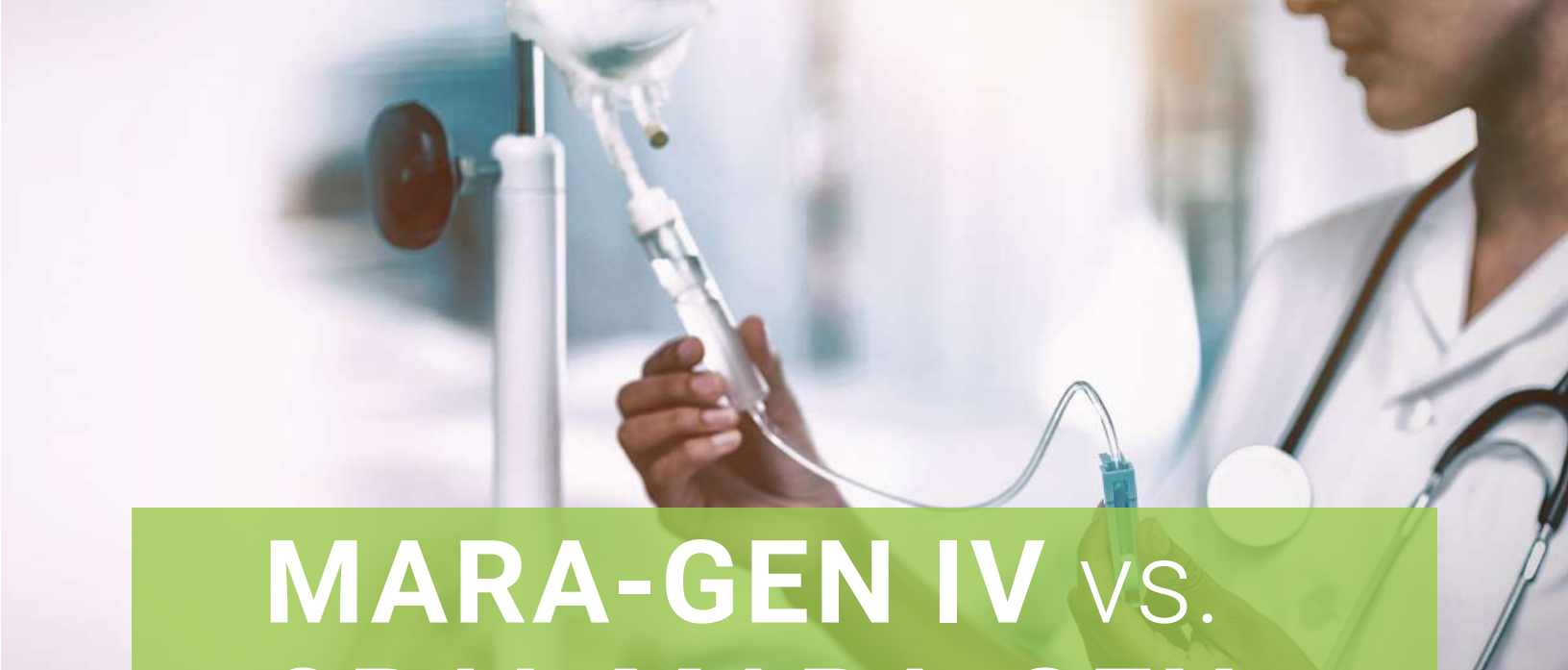
AIC CALCIUM CARBONATE (BREAKS DOWN TO SAFE COMPONENTS)



REGULAR CALCIUM CARBONATE



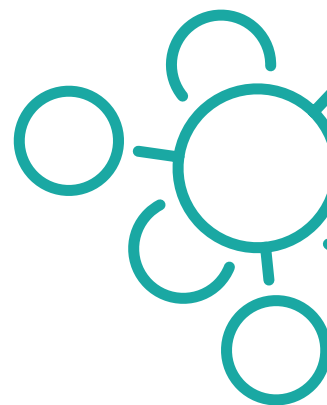
* Equilibrium Constant



MARA-GEN IV vs. ORAL MARA-GEN

5-10 ml of Mara-Gen IV is mixed into 250 ml of either sterile or 0.9% saline solution bag and dripped for about an hour. Other common IV infusions such as high dose vitamin C, magnesium chloride, curcumin, etc. can be mixed in and infused together without any contraindications. Because AIC is infused directly into our system, Mara-Gen IV circumvents the absorption challenges of oral Mara-Gen through the stomach lining (mucosa), and thus a stronger dose of AIC can be given safely.

When 10 ml of AIC solution is given, up to 16 mg of ionic calcium is introduced in our body, and the efficacy is raised significantly higher for patients who may have more significant calcium needs. Such a high dose is, however, not necessary for patients with low calcium needs because our body can utilize only so much ionic calcium, and the excess is excreted to bring the ionic calcium level back to the average level within approximately 4 hour-period.





MARA-GEN IV DOSING PROTOCOL

Form & Ingredients:

Liquid in 120ml bottle similar to oral Mara-Gen. Identifiable by a blue dot on top of the product package.

Mara-Gen IV contains only AIC calcium carbonate, without selenium, zinc, and magnesium.

Usage:

Open and close the bottle as quickly as possible to minimize CaCO_3 formation. The solution needs to be filtered with syringe filter (0.22 μm , hydrophilic) when added to an IV solution bag. 10-15ml of AIC can be administered to 250 ml saline or sterile solution and dripped for an hour.

CKD patients can use a 100ml IV solution dripped for an hour. Minimum of two IV sessions per day with 4-6 hours apart for cancer patients are recommended. Adding one or two oral doses of Mara-Gen may increase the efficacy. Syringe filter (0.22 μm , hydrophilic) should be used to filter out precipitated calcium carbonate as it could irritate the vein. Precipitation is normal and does not indicate reduced potency.

Procedure:

1. Open the bottle and draw out AIC with a syringe which has a syringe-filter attached to filter out larger particles. The filter need not be sterile packaged as AIC is sterile. Close the bottle tightly right away.
2. Inject it into an IV bag and drip for one hour.



NEBULIZING MARA-GEN

AIC can also be inhaled through a nebulizer. AIC's property is still intact, and only about 15-20% of a dose of AIC (5 ml) can be absorbed through the epithelial lining of the nasal cavity and lungs. Nebulizing is only recommended for supplementing either oral or IV AIC protocol for diseases that can benefit further from nebulizing, such as tumors or cysts in the nasal cavity, lungs, or ear canal where the absorption of AIC can directly affect the disease. Nebulizing can also help COPD, bronchitis, pneumonia, asthma, and cystic fibrosis.

However, nebulizing alone will not be enough for the treatment, but only to supplement oral or IV AIC treatment. Pneumonia should be treated together with antibiotics and not replace it.

Nebulizing Dosage:

5 ml of Mara-Gen in 50ml of water. Mara-Gen to water ratio is 1:10. For nebulizing, either Mara-Gen IV or oral Mara-Gen can be used. Any water suitable for oral intake can be used for nebulizing. Do not use a saline solution.

A portable nebulizer only holds 10-20ml of solution at a time. Use a bigger machine or repeat multiple times to nebulize all 50ml per session. At least two sessions a day are recommended for severe conditions.



AIC DOSING FOR PREVENTIVE HEALTH



To trigger physiological responses that turn on thyroid stimulating hormone (TSH) and calcitonin, which activates osteoblasts involved in bone-building process, about 3 mg dose of AIC seems to be enough for typical healthy adults. AIC provides 5-8mg of ionic calcium depending on the products used, which triggers robust bone building not possible for most elderly people. For example, there is enough AIC in one dose of Mega-Gen (our maintenance product providing 5 mg of AIC in 5 ml solution) to help maintain good calcium homeostasis and protect bone health for healthy adults.

Bone health is a good indicator of overall health as many research has shown. By strengthening the bone turnover process, AIC prevents age-related bone loss, which starts around the average age of 35. This natural bone loss, often exacerbated by poor diet, unhealthy lifestyle, hormonal imbalance, accumulating toxins, and poor gut health, lay a pathogenic pathway for most degenerative diseases.

One dose of Mega-Gen a day, therefore, serves as proper maintenance and preventative dose for the healthy individuals who want to maintain good bone health.



AIC DOSING FOR DEGENERATIVE DISEASE



Chronically sick patients have much higher calcium needs due to the presence of an active disease momentum. In all cases, there is a profound ionic calcium deficiency and broken calcium homeostasis for unhealthy patients. Therefore, for such patients, more AIC is needed per dose and per day to address deep calcium needs and to counteract the disease momentum.

To bring in faster systemic change and to assure a better chance for recovery, a higher dosage from the beginning of the treatment is recommended. Mara-Gen, which provides 8 mg of AIC in 5ml solution per dose, is prescribed for patients with sickness, often 3 times a day, to keep the healing momentum going. A patient can take AIC every three to four hours, even up to six times a day, if doable. The recommended length of the therapy is three to six month to see favorable results.

Robust bone rebuilding process triggered by AIC helps to reverse cellular calcification, to reverse mitochondrial dysfunction, to restore cellular signaling, and to reduce oxidative stress, thus reducing inflammation and triggering health restoration. AIC also normalizes body pH to ideal 7.4 and modulates and corrects calcium signals in immune cells to work properly to combat autoimmune responses. Patients with an advanced stage of the disease with other concurrent calcium-related diseases may take longer to see the desired effects of AIC therapy, however.

NEURODEGENERATION DOSING **PROTOCOL**

For Parkinson's, MS, ALS: Use Mara-Gen | For Alzheimer's: Use Alzi-Gen.

Dual X-ray absorptometry (DEXA) or Radiofrequency Echographic Multispectrometry (REMS) ultrasound bone scans are also recommended at the beginning of each phase to measure bone health progress. Reaching the minimum T score of -1.0 (normal) is recommended before moving onto Phase 3 if the patient started with low bone density.

PHASE 1



Take 5ml of Mara-Gen
in 500ml of water
3x daily

PHASE 2



Take 5ml of Mara-Gen in
500ml of water
2x daily

PHASE 3



Take 5ml of Mara-Gen
in 500ml of water
1x daily

Post-Phase 3 Maintenance Dosage: 1x Mara-Gen daily

Notes:

In the case of Alzheimer's disease treatment, please replace Mara-Gen with Alzi-Gen. Follow the same dosing protocol.



ARTHRITIS DOSING PROTOCOL

For Osteoarthritis & Rheumatoid Arthritis

AIC is not recommended for bone-on-bone arthritis cases.

DEXA/REMS bone scans are also recommended at the beginning of each phase to measure bone health progress. Reaching the minimum T score of -1.0 (normal) is recommended before moving onto Phase 3 if the patient started with low bone density.

PHASE 1



+



PHASE 2



Take 5ml of Mara-Gen in
500ml of water
2x daily

Take 5ml of Carti-Gen
in 500ml of water
1x daily

Take 5ml of Carti-Gen
in 500ml of water
2x daily

**Post-Phase 2
Maintenance Dosage:**

1x Carti-Gen daily



OSTEOPOROSIS DOSING PROTOCOL

To benefit from AIC, patients must stop osteoporosis drugs and wait 3-6 months before taking AIC.

Cancer patient's bone density will not rise much until remission happens first.

DEXA/REMS or C-telopeptide of type I collagen (CTx) & Procollagen Type I Intact N-terminal Propeptide (P1NP) blood bone markers are also recommended at the beginning of each phase to measure bone health progress. Reaching the minimum T score of -1.0 (normal) is recommended before moving onto Phase 3 if the patient started with low bone density.

PHASE 1



Take 5ml of Mara-Gen in 500ml of water
3x daily

PHASE 2



Take 5ml of Mara-Gen in 500ml of water
1x daily

PHASE 3



Take 5ml of Mega-Gen in 500ml of water
1x daily

Phase 3 Maintenance Dosage: If T score is below -1 at the end of Phase 2, staying on Phase 2 until the T score reaches -1.0 before moving onto Phase 3 is recommended.

Notes:

DEXA measures bone mineral density (BMD) only and is not a good indicator for bone quality and strength. People with the same T-score could have huge difference in fracture risk. People with low bone quality or who have been taking antiresorptive drugs may experience loss of BMD first before rising again. Wait 3-6 months after stopping antiresorptive drugs to begin with AIC. Bone building requires good macro and micro-nutrients, normal iodine level, and normal hormone levels.

DENTAL DOSING PROTOCOL

Removal of infected tooth and toxic tooth by a qualified biological dentist is recommended. When taking AIC for dental purposes, swish around in the mouth before swallowing each mouthful, as AIC is absorbed through epithelial lining of mouth as well.

DEXA/REMS bone scans are also recommended at the beginning of each phase to measure bone health progress. Reaching the minimum T score of -1.0 (normal) is recommended before moving onto Phase 3 if the patient started with low bone density.

PHASE 1



+



Take 5ml of Mara-Gen in
500ml of water
2x daily

Take 5ml of Denti-Gen
in 500ml of water
1x daily

Take 5ml of Denti-Gen
in 500ml of water
2x daily



**Post-Phase 2
Maintenance Dosage:**

1x Denti-Gen daily



RECOMMENDED TESTS FOR OSTEOPOROSIS

The X-ray based bone scan known as DXA or DEXA Bone Density Test can be used for before and after AIC treatment comparison. However, since DEXA cannot measure bone quality and strength, DEXA results may not be the best indicator for measuring the efficacy of AIC therapy which works by repairing and replacing bones. (turnover) People with the same T-score could have huge difference in fracture risk. People with low bone quality or who have been taking antiresorptive drugs may experience loss of BMD first before rising again. Wait 3-6 months after stopping antiresorptive drugs to begin with AIC.

For patients who do not want radiation exposure, there are serum bone bio-marker tests available: CTx and P1NP. It is best to measure two months after the treatment.

CTx bone resorption marker: C-telopeptide assesses the rate of bone turnover, which can be particularly useful for older individuals. Measures osteoclast function.

Optimal: <400 pg/mL

Danger Zone: >600 pg/mL

P1NP bone formation marker: N-Terminal Propeptide of Type 1 Procollagen measures bone formation. Measures osteoblast function.

Without pharmaceuticals: 20-100 µg/mL

Pharmaceuticals will change the results.

The urine NTx (N-telopeptide) bone test is inexpensive and non-invasive, making it easier to perform, but serum CTx is preferred. Serum CTx tends to change more than urine NTx. Nevertheless, both are useful and currently in use.

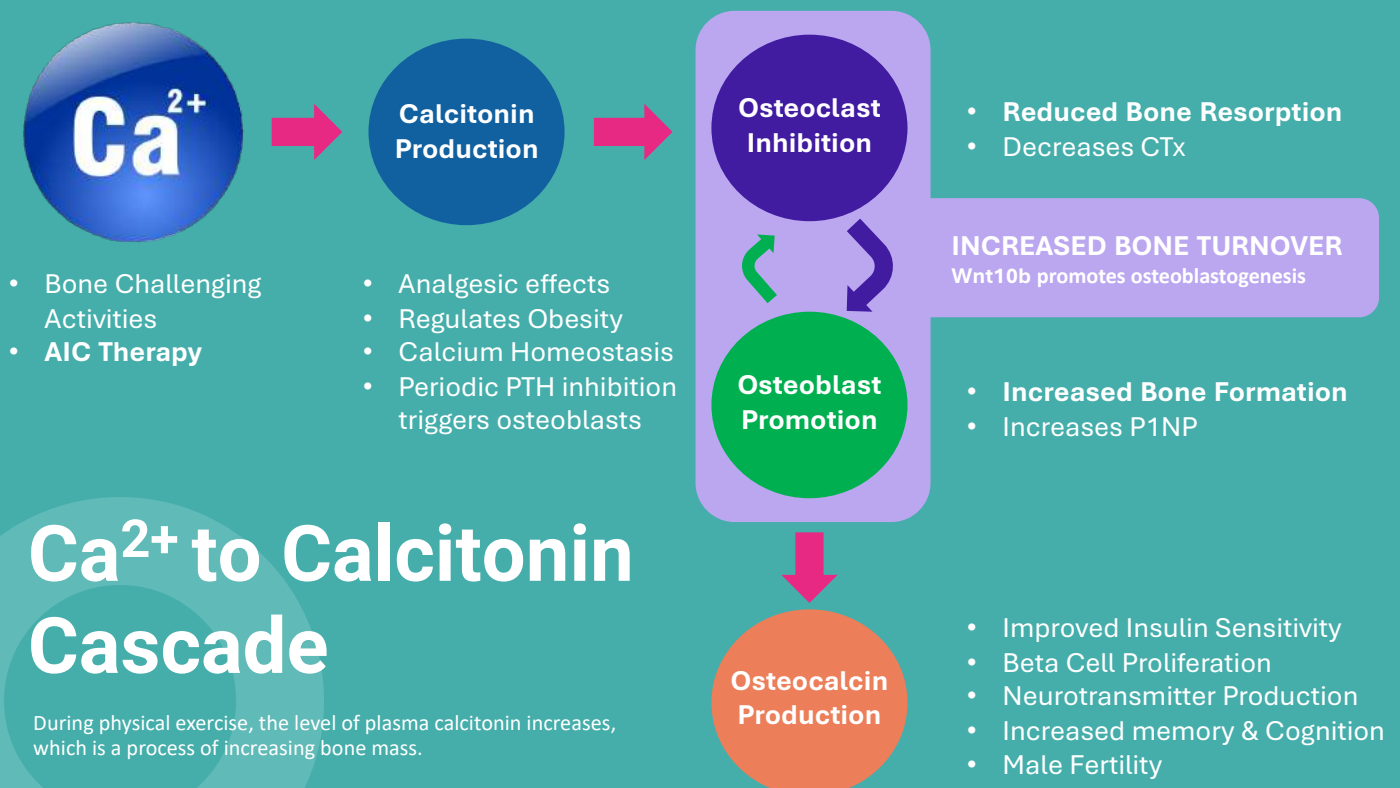
ENDOGENOUS CALCITONIN BONE-BUILDING

Calcitonin is a peptide hormone, triggered by elevated ionic calcium level, which inhibits osteoclasts and also triggers osteoblast cells that rebuilds bones.

Exogenous calcitonin treatment for bone health (derived from salmon), although considered the best agent to rebuild osteoporotic bones, failed to be the first line of treatment due to difficulty in administration and side effects. Despite not being a strong anabolic treatment for bones, it is still used in certain medical situations for its anti-resorptive and other properties.

AIC therapy, for the first time in medical history, perfected calcitonin based bone-building by triggering endogenous calcitonin without the use of cumbersome animal calcitonin. This is better than just 'keeping' old bones by using antiresorptive drugs.

New research has found that there is no age-related decrease of plasma calcitonin concentrations, calling into question the concept that a progressive deficiency of calcitonin is partly responsible for age-related osteoporosis. It is age-related ionic calcium deficiency that limited the release of endogenous calcitonin. AIC now removed the barrier.





DEXA vs. REMS

Radiation Based
Dual Energy
X-ray absorptiometry

Ultrasound Based
Radiofrequency Echographic
Multi Spectrometry

AIC THERAPY & DEXA MEASUREMENTS

Did you know why antiresorptive drugs (bisphosphonates and denosumab) have a warning of rapid bone loss after stopping the medication? Because the drugs work by slowing down bone turnover, it essentially stops everyday bone repairs to keep an old bone. Many years of use of these drugs will severely compromise bone health. Upon stop using these drugs, our body's natural bone turnover process will try to remove poor quality dead bone mass. Therefore, rapid loss of bone mineral density (BMD) can be experienced.

AIC works by upregulating bone turnover by endogenous calcitonin release, which works by remodeling and repairing while rebuilding it. Therefore, DEXA results (BMD and T-score) may drop initially for some patients if their bone quality was poor or have been on antiresorptive drugs.

Patients with poor bone quality or who have been taking strong anti-resorptive agents may experience worsening DEXA results within 12 months of using AIC therapy as body clears dead bone mass before rebuilding healthy bone matrix. Meanwhile, doing CTx and P1NP serum tests will verify that AIC is rapid repairing damaged bones, increasing bone quality and strength and reducing fracture risks. Instead of DEXA, we also recommend using ultrasound based REMS which gives bone quality readings.

BONELOSS & DEGENERATIVE PATHOPHYSIOLOGY

IONIC CALCIUM DEFICIENCY

Either Not Getting it Enough or
Unable to Utilize Existing Protein Calcium



PROCESSED FOODS

Stored calcium is leached out from the bones to restore pH balance as buffer



LESS ACID IN STOMACH OVER 60 YEARS OLD

Weak acid in stomach for age 60+ leads to less calcium absorption



LACK OF EXERCISE / SEDENTARY LIFESTYLE

Absorbed calcium doesn't end up in the bones since only weight bearing exercise triggers bone formation



STRESS/ FEAR

Unrelieved stress causes inflammations that uses up calcium.

"Of course, all of these diseases may have many causes, but they also all have one thing in common - calcium deficiency."

- Dr. Robert Heaney, M.D.

"Such subtle dysregulation of Ca^{2+} signals have been linked to some of the major diseases in humans such as cardiac disease, schizophrenia, bipolar disorder and Alzheimer's disease."

Calcium signaling remodeling and disease Michael J. Berridge,
Biochemical Society Transactions



AUTOIMMUNE & INFECTIOUS DISEASE DOSING PROTOCOL

For Lupus, Crohn's, Hashimoto's, Eczema, Celiac, Vitiligo, etc

For Lyme Disease, long COVID, HIV, etc.

Please take AIC on an empty stomach at least 30 minutes before a meal.

DEXA/REMS ultrasound bone scans or CTx & P1NP blood bone markers are also recommended at the beginning of each phase to measure bone health progress. Reaching the minimum T score of -1 (normal) is recommended before moving onto Phase 3 if the patient started with low bone density.

PHASE 1



Take 5ml of Mara-Gen
in 500ml of water
3x daily

PHASE 2



Take 5ml of Mara-Gen
in 500ml of water
2x daily

PHASE 3



Take 5ml of Mara-Gen
in 500ml of water
1x daily

Post-Phase 3 Maintenance Dosage: 1x Mara-Gen daily

Notes:

For infectious disease, AIC has properties to inhibit pathogen proliferation. However, AIC will not replace antibiotics. AIC fights inflammation and chronic symptoms associated with infection. It is recommended to reduce the use of steroid as symptoms improve.

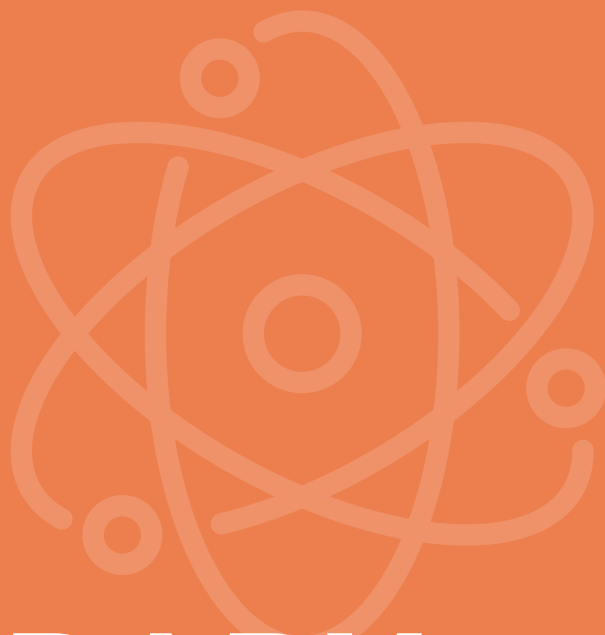


3 MONTHS OF MINIMUM TREATMENT

Because of AIC's workings from cellular to systemic levels, the health improvement experienced may take time and the results may fluctuate for the first 3-month period of the therapy for most patients, until some positive results start to dominate.

It is also during this first 3-month period of the treatment when healing reactions are experienced for some patients as our body tries to balance the healing effects of AIC. Healing reactions from AIC tend to subside and disappear for most people during the second 3-months period of AIC therapy.

Therefore, it is advised that AIC treatment should be given at least for a minimum of 3-month period to look for any positive results before committing another 3-month of AIC therapy. If any positive results are observed during the first 3-month period, then we recommend another 3-month period of therapy to experience more accelerated positive effects. The second 3-month period is a sweet spot for most patients.



AIC THERAPY FOR CANCER

AIC therapy had many successes with all staged cancer patients, including the last stage with metastasis alongside physicians comprehensive therapy. AIC therapy comes as an excellent adjunct and adjuvant therapy working synergistically alongside many alternative therapies without contraindications. AIC therapy, however, is not very useful if administered alongside aggressive conventional cancer treatments as it fights the side effects more than the cancer itself.

Some patients respond very well to AIC therapy and may produce improved blood works and heightened energy and activity levels. However, sometimes such rapid recovery may produce contradicting medical results such as enlarged tumor size and elevated cancer markers. In such cases, as long as the patient's condition is better and all the blood works look promising, please do not stop AIC therapy as enlarging tumor size and elevated markers also indicate tumor necrosis/lysis.

Rapid apoptosis of cancer tumors may cause cancer to look more prominent in the scans and raise the cancer markers in the initial weeks to even alarming level. In many clinical cases, enlarged tumors collapsed and cancer markers dropped as the patients' condition continued to improve. Some tumors were left hollow with scar tissues and began to breakdown subsequently. Please follow the AIC therapy phases from 1 to 3 for all cancer stages and monitor the progress carefully under the care of physicians involved.





AIC DOSING FOR CANCER

Most cancer patients display low skeletal bone density and heavy calcification both in systemic and intracellular levels, which indicate severe calcium displacement caused by disturbed calcium homeostasis. Also, cancer patient's low body pH caused by lactic acid production of cancer cells further indicates the deep need for ionic calcium as it plays a critical role in maintaining proper body pH and to maximize oxygen delivery.

Because cancer patient's physiological need for ionic calcium is 2 to 5 times greater than healthy people, a minimum of 4 to ideal 5 to 6 daily doses of Mara-Gen is recommended for all stage cancer patients, maximizing the body's chance against to cancer. Good time to take AIC if taking four times a day is right after rising up, 30 minutes before lunch, 30 minutes before dinner, and one to two hours before bed, giving 3~4 hour gap between each doses.

AIC therapy is effective against cancer for the following changes to health outcomes:
(1) Restores body to the ideal pH by neutralizing lactic acid and ammonia, which turns on the immune response around tumors, inhibits angiogenesis to limit feeding cancer cells, and maximizes oxygen supply
(2) Restores the function of p53 gene and induces apoptosis
(3) Inhibits inflammatory enzymes, such as COX-2, which spreads cancer
(4) Restores mitochondrial functions and builds a strong barrier of healthy cells around cancer
(5) Reduces oxidative stress, systemic inflammation, and autoimmune responses, providing anticancer physiological environment.



RECOMMENDED AIC DOSING PROTOCOL

All Stage Cancers:

Please take AIC on an empty stomach at least 30 minutes before a meal. Take one dose after waking up, one at 30 minutes before lunch, one at 30 minutes before dinner, and one at 1-2 hours before bed, putting 3~4 hour gap between each doses.

Blood panels and markers are recommended for every 2-4 weeks to monitor the progress.

DEXA/REMS bone scans or CTx & P1NP blood markers are also recommended at the beginning of each phases to measure bone health progress. Reaching the minimum T score of -1.0 (normal) is recommended if the patient started with low bone density.

PHASE 1



Take 5ml of Mara-Gen in
500ml of water
4x daily

PHASE 2



Take 5ml of Mara-Gen
in 500ml of water
3x daily

PHASE 3



Take 5ml of Mara-Gen
in 500ml of water
2x daily

Post-Phase 3 Maintenance Dosage: 1x Mara-Gen daily after 5 years of no recurrence

Notes:

The number of bottles required is based on 250ml size. In the above estimate, Phases 1, 2, and 3 are estimated to be 6 months each. Please refer to cancer dosage protocol to manage expectations.

AIC THERAPY PHASE ONE

Phase 1: Remission Induction

Duration: Minimum 6 months | **Dosing:** 4-6 daily doses

Many positive results or rapid recovery from symptoms may be experienced during Phase 1. Results may fluctuate for the first three months, but if AIC therapy seems to be helping in any way, continue for another three months. When the patients feel better, and symptoms start to disappear, do not stop or reduce the dosage of the protocol but maintain at the protocol's max dosage.

At this phase, the improved condition is maintained only by the continued cellular modulation of AIC. The purpose of Phase 1 is reversing the disease momentum and rebuilding systemic health.

There is a high chance of recurrence of disease (especially cancer) if AIC protocol stops here. Continue Phase 1 until full remission. Adjuvant detox programs are recommended during Phase 1 based on the patient's toxin profiling.

PATIENT'S HEALTH CONDITION

DISEASE

OWN HEALTH

AIC SUPPORT

In Phase 1, AIC supports and strengthens compromised health of patients to bounce back. Continue Phase 1 until full remission or the disease may overtake again.

AIC THERAPY PHASE TWO

Phase 2: Consolidation

Duration: Minimum 6 months | **Dosing:** 2-3 daily doses

Phase 2 starts when symptoms have completely disappeared, and patients feel already recovered from the disease. If the purpose of Phase 1 is to turn the disease momentum around and overcome the disease symptoms, the purpose of Phase 2 is to give our body enough time to repair and strengthen our own health to eliminate the chance of recurrence. Strong immune system can help wipe out remaining hidden pathogenic cells.

It took years for our health to slowly fail and succumb to disease, and therefore we must give our body sufficient time to rebuild it. Since bone health is closely related to cellular calcification and subsequent cellular damages, taking AIC therapy at full dose (3x daily) all the way until the bone density returns to the normal range is recommended. (*Preferably until T-score greater than -1*) Research has shown that poor bone health increases over all mortality over 30%. AIC therapy has many health benefits.

PATIENT'S HEALTH CONDITION

OWN HEALTH

AIC SUPPORT

In Phase 2, AIC supports and strengthens patient's cellular and systemic health not to be succumbed recurrence. Continue Phase 2 until all the benefits of AIC therapy are fully realized.

AIC THERAPY PHASE THREE

Phase 3: Maintenance

Duration: 5 years | **Dosing:** 1-2 daily dose

Phase 3 starts when patient is fully recovered from disease symptoms and cleared from all medical examinations. Because of AIC therapy's efficacy on a wide range of health spectrum, the goal of AIC therapy is to help patients enjoy better health than when disease symptoms first appeared. The purpose of Phase 3 is to maintain what has been achieved through Phase 2.

As we age, cellular calcification from bone loss, which leads to cellular dysfunction, is inevitable. It is considered as one of main trigger of degenerative diseases. AIC therapy's maintenance program, Phase 3, together with a healthy diet and lifestyle, can prevent diseases and maintain optimal health. 1-2 dose/day of AIC is recommended for recovered patients. Minimum 5 years of maintenance is recommended for patients recovering from cancer.

PATIENT'S HEALTH CONDITION

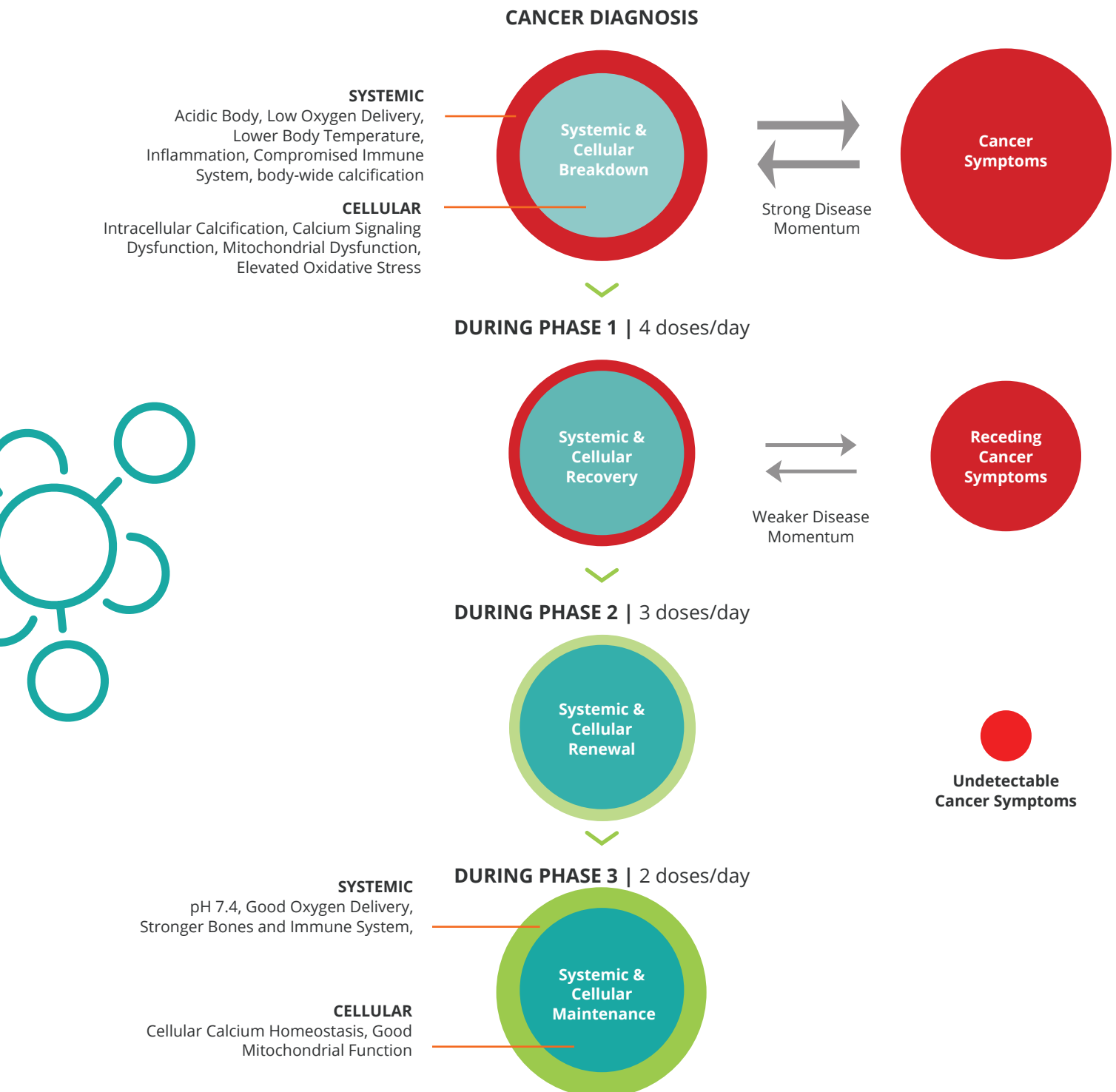
OWN HEALTH

AIC

In Phase 3, AIC supports and strengthens patient's cellular and systemic health not to be succumbed to recurrence and lead a healthier life. AIC is a great preventive measure.

AIC THERAPY RECOVERY PHASES

Even after successful conventional treatments with the absence of cancer symptoms, AIC therapy Phase I is recommended if the underlying cancer-causing condition is not addressed. Some aggressive treatments with collateral damage may leave patient's health more compromised, raising the risk of recurrence.

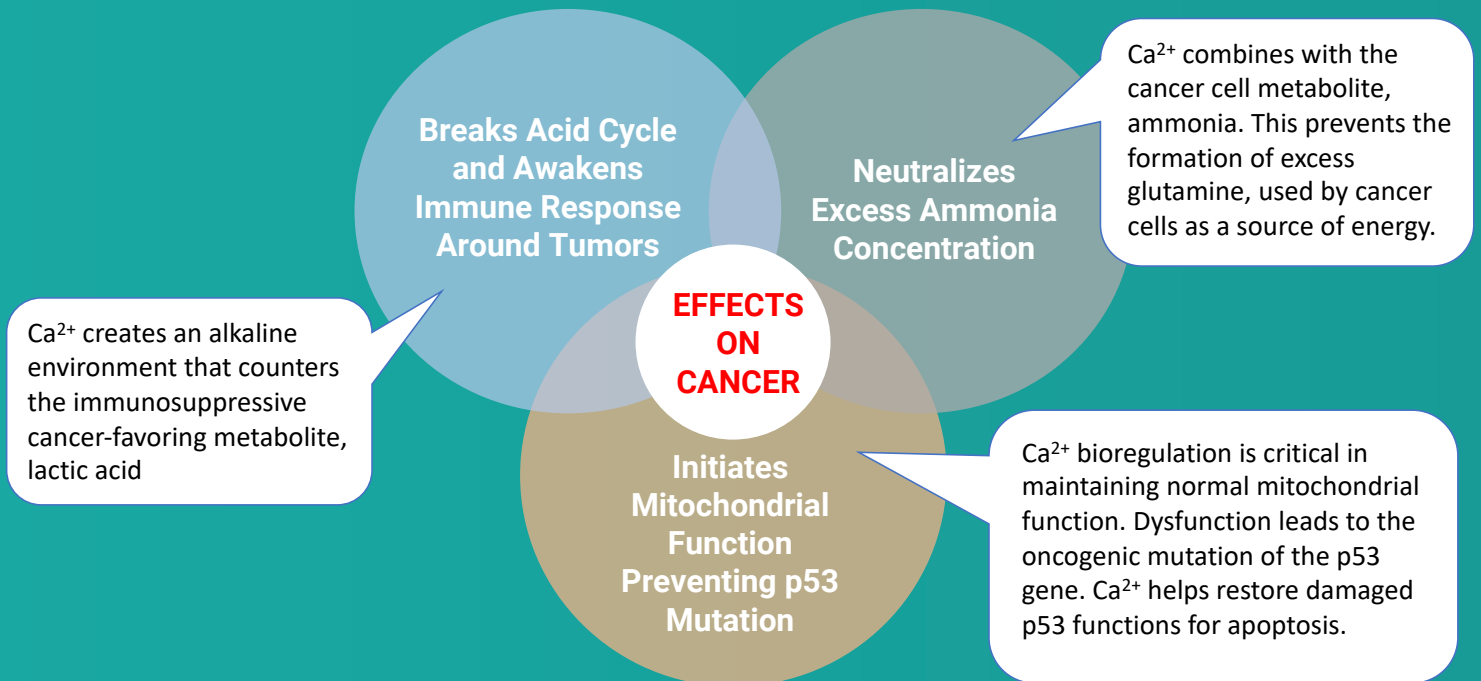


MULTI-FACETED ASSAULTS ON CANCER

Calcium ion (Ca^{2+}) is a ubiquitous and versatile signaling molecule controlling a wide variety of cellular processes, such as proliferation, cell death, migration, and immune response, all fundamental processes essential for the establishment of cancer.

Altered Ca^{2+} transmission has been implicated in a variety of processes fundamental for the uncontrolled proliferation and invasiveness of tumor cells and other processes important for cancer progression.

Normalizing calcium homeostasis from cellular to systemic level has manifold effects against cancer by creating adverse environment for cancer growth and metastasis.



Dr. Carl J. Reich considered cancer as the ultimate adaptation to ionic calcium deficiency, "tailor-made" to survive and to thrive in an ionic calcium deficient environment.

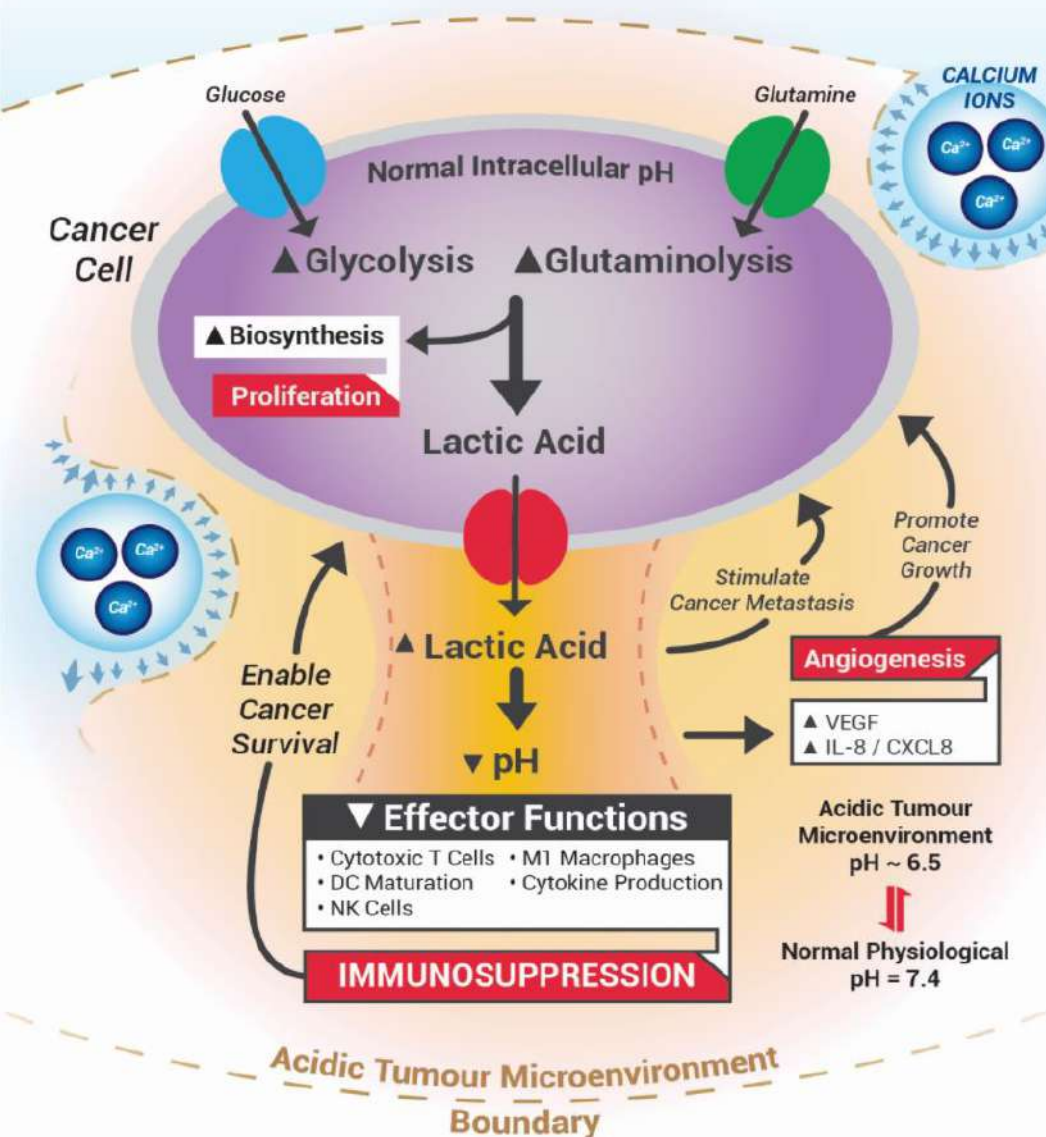
(The 'Calcium Factor' by Dr. Carl J. Reich, M.D.)

AIC THERAPY EXPOSES CANCER TO IMMUNE SYSTEM

NEUTRALIZATION OF LACTIC ACID IN CANCER BY Ca^{++}

LACTIC ACID

CALCIUM LACTATE

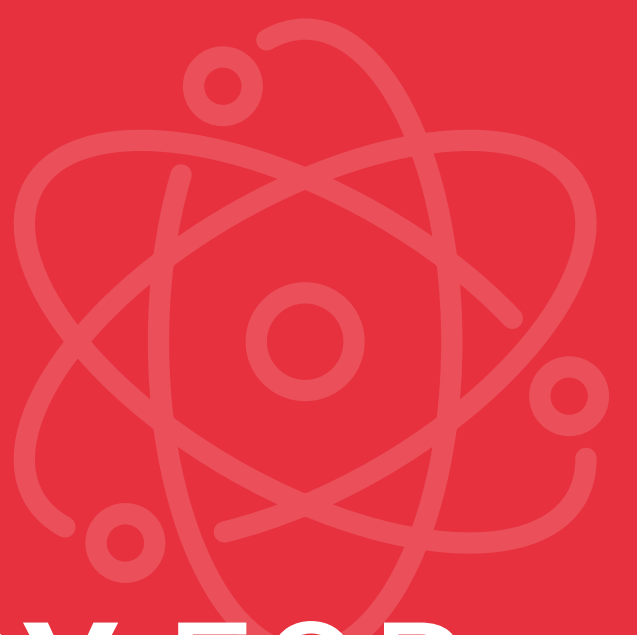


Cancer cells can enhance their survival by suppressing the anti-cancer immune response by actively maintaining a slightly acidic micro-environment (pH 6.0–6.5) which has been reported to lead to loss of T-cell function and serves as a basis for the establishment of the malignancy and its progression.

They do this by secreting lactic acid by altering their metabolism, and high tumor lactate level is predictive of metastasis and restricted patient survival. Therefore, cancer-generated lactic acid is a critical, immunosuppressive metabolite rather than a 'waste product'. This paradigm shift can have major impact on therapeutic strategy development.

"By tracking the chemical transformations of lactate within cancer cells, the researchers found that, rather than being expelled from cells, lactate was being taken up by mitochondria, which are considered the energy factories of cells."

Using new metabolomics technologies, the researchers found that cancer cells can use the compound lactate to fuel biochemical reactions and to generate other compounds needed for cell growth, such as lipids to build new cellular membranes."



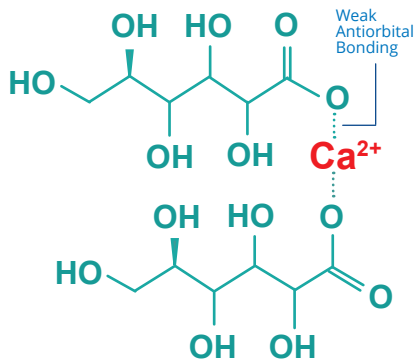
AIC-SG THERAPY FOR CANCER

Cancer cells have significant heterogeneity in glucose metabolism. Most cancer cells rely largely on aerobic glycolysis as it accounts for 56–63% of their ATP budget. So, cancer cells plunder more glucose from microenvironment up to 200x more than normal cells and secrete more lactic acid to meet requirement of energy and material metabolism. This characteristic is utilized in PET scans to detect cancer by attaching radioactive isotope of fluorine to glucose. This idea of using glucose as Trojan horse molecule to insert other molecules into pathogenic cells has been utilized in cancer treatment.

CBHI invented another molecule based on calcium gluconate, unlike AIC which is based on calcium carbonate. It is named AIC-SG, where 'S' stands for 'stealth' and 'G' stands for 'glucose'. Because of antiorbital chemical bonding, our body sees AIC-SG as just glucose and when taken on an empty stomach, glucose hungry cancer cells gobble up most of AIC-SG through GLUT. In oxygen deficient environment of tumor cells, AIC-SG produces free radicals that kills cancer cells.



INVENTION OF AIC-SG MOLECULE

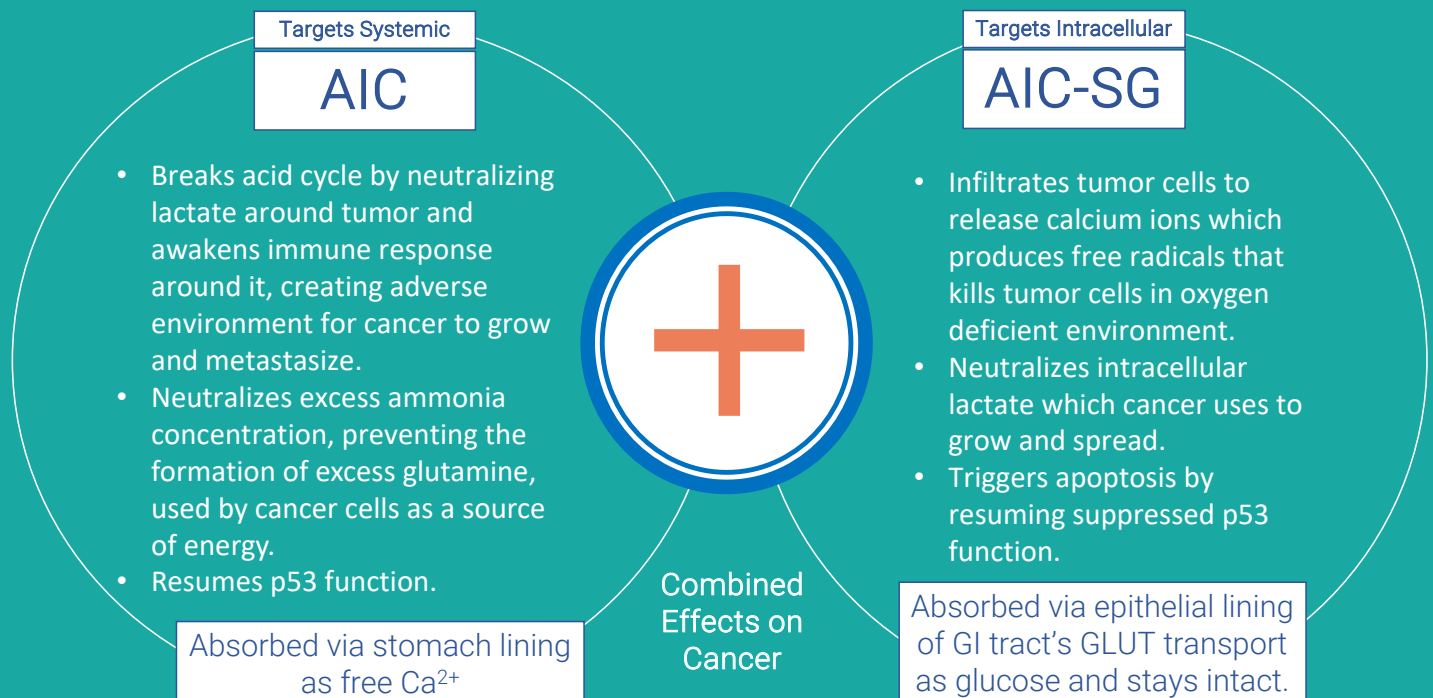


Manufactured with Antiorbital Formulation Technology, AIC-SG embeds antiorbital calcium within calcium gluconate. Hidden AIC calcium is invisible to our body and AIC-SG is perceived as glucose, not calcium gluconate. (Stealth Glucose)

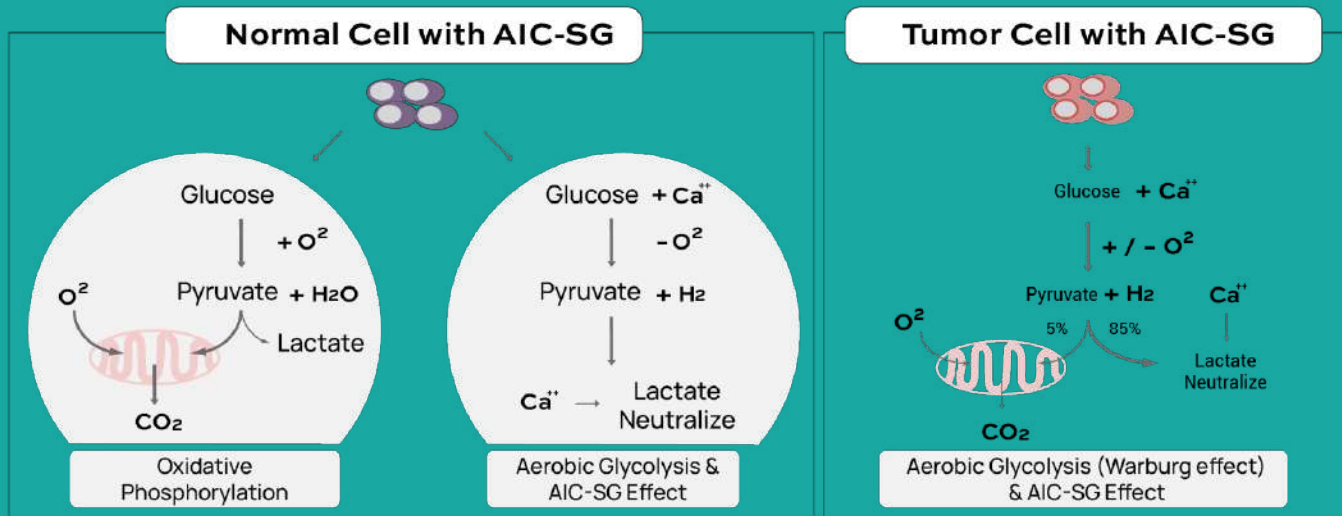
AIC-SG contains AIC 150mg/5ml which is almost 20 times more ionic calcium intake than 5ml of AIC (Mara-Gen). Taken together with AIC three times a day, it puts about 480mg of ionic calcium in our body each day.

Whereas AIC releases ionic calcium into arteries and triggers systemic calcium homeostasis by raising serum ionic calcium level by 2-3%, AIC-SG releases ionic calcium intracellularly as AIC-SG is metabolized inside the cells.

AIC + AIC-SG COMBINATION THERAPY

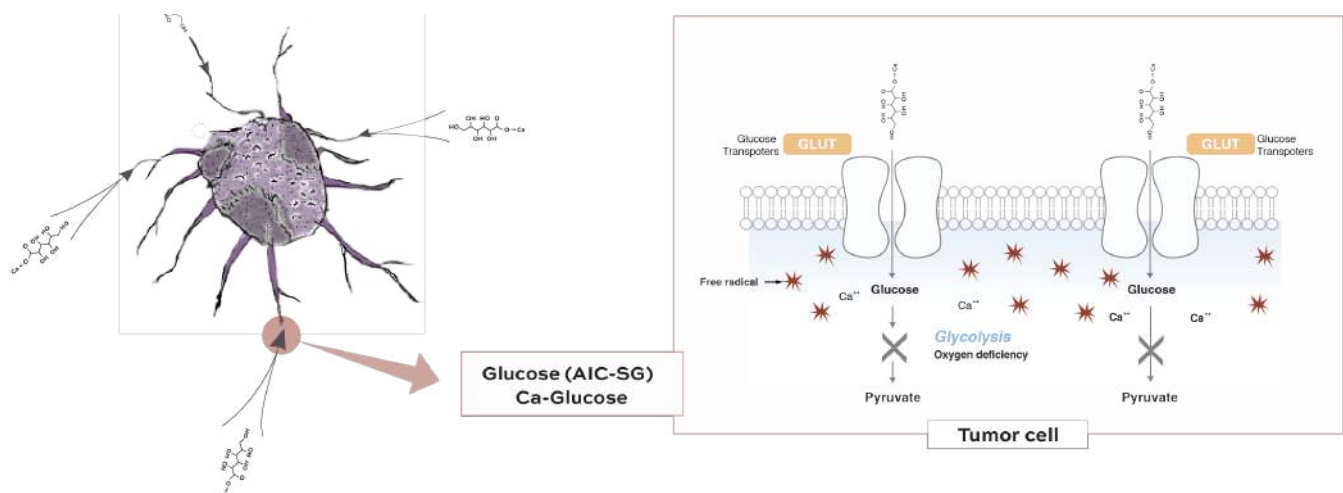


SELECTIVE TOXICITY OF AIC-SG MOLECULE



In cancer cells, calcium ions released from metabolized AIC-SG coming through GLUT result in excess calcium ions, which triggers free radical production and causes the cancer cell to die from oxidation. In normal cells that are in an oxygen-rich state, free radicals do not occur. Also, excess calcium ions neutralize lactate produced within cancer cells, stifling its ability to grow and metastasize.

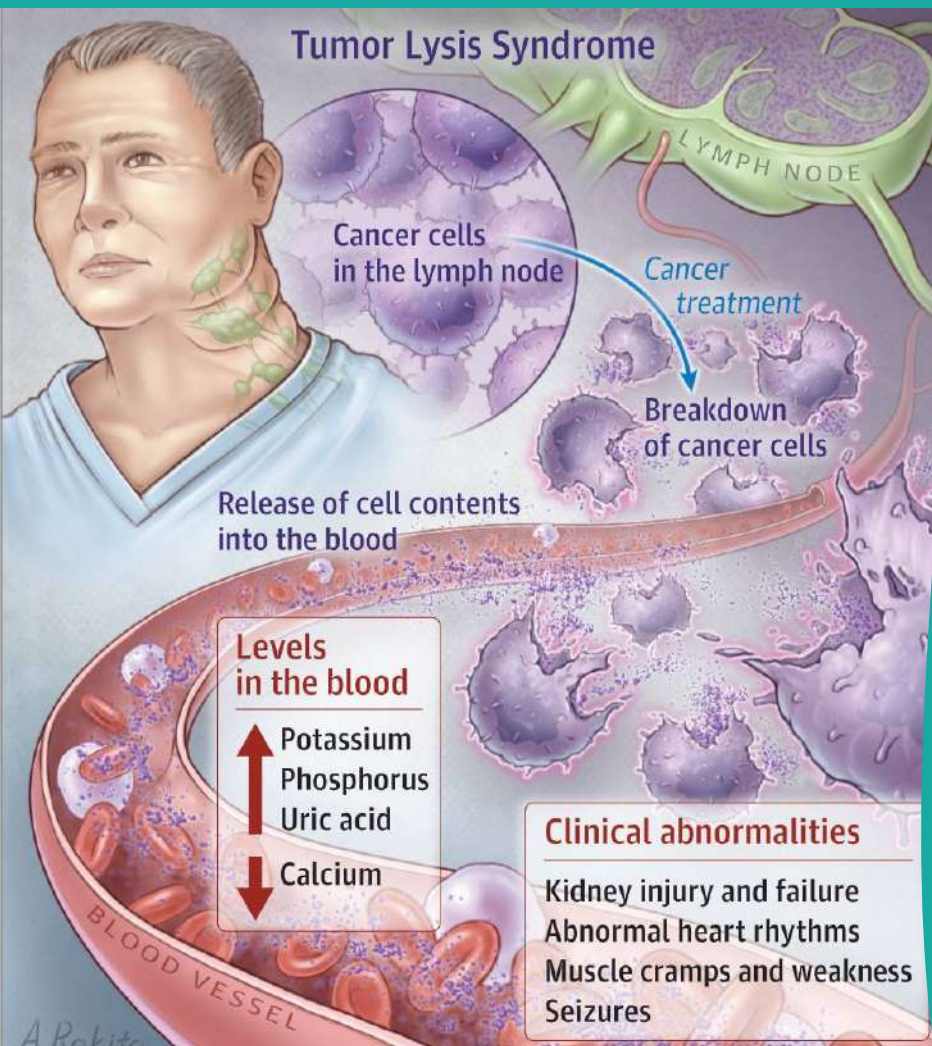
Elevated calcium levels contribute to apoptosis activation by modulating the activities of several proteins, including CAMK, calcineurin, calpain, and PKC. These proteins participate in various signaling pathways and cellular processes, ultimately leading to cell death when calcium concentrations become excessive.



TUMOR LYSIS SYNDROME MONITORING

In cancer treatments, patients may experience electrolyte disturbance when tumor cells die out faster than the body can remove the debris of dead cells. It is always advised to monitor for TLS for patients undergoing AIC + AIC-SG therapy especially combined with other cancer treatments. Tumor lysis may make tumors look enlarged and cancer markers spike high.

TLS: Uric acid level is monitored within two weeks of therapy to minimize TLS. With good kidney function and GFR, 480 $\mu\text{mol/L}$ (8 mg/dL) of uric acid level is often considered a concern and may warrant intervention. Blood drop monitor with strips is recommended over urine strips.



Clinical symptoms include:

Nausea, irregular heart rate, confusion, restlessness, irritability, delirium, hallucinations, seizures, vomiting, diarrhea, muscle cramps, twitches, weakness, numbness or tingling, fatigue, decreased urination.

Laboratory Diagnosis of Tumor Lysis Syndrome

Requires 2 or more of the following criteria achieved in the same 24-hour period from displaying symptoms of TLS.

- Uric acid 25% increase from baseline or greater than or equal to 8.0 mg/dL
- Potassium 25% increase from baseline or greater than or equal to 6.0 mg/L
- Phosphorus 25% increase from baseline or greater than or equal to 4.5 mg/dL (greater than or equal to 6.5 mg/dL in children)
- Calcium 25% decrease from baseline or less than or equal to 7.0 mg/dL

RECOMMENDED AIC-SG DOSING PROTOCOL

All Stage Cancers:

Please take AIC + AIC-SG on an empty stomach at least 30 minutes before a meal.

Take one dose after waking up, one at 30 minutes before lunch, one at 30 minutes before dinner, or one at 1-2 hours before bed.

Blood panels and markers are recommended for every 2-4 weeks to monitor the progress.

DEXA/REMS bone scans or CTx & P1NP blood markers are also recommended at the beginning of each phases to measure bone health progress. Reaching the minimum T score of -1.0 (normal) is recommended.

PHASE 1



Take Both 5ml of Mara-Gen & 5ml of AIC-SG in 500ml of water
3x daily

(Until symptoms disappear.
Watch for TLS and adjust
AIC-SG dosing)

PHASE 2



Take 5ml of Mara-Gen in 500ml of water
3x daily

(Recommended for 3-6 months after Phase 1)

PHASE 3



Take 5ml of Mara-Gen in 500ml of water
2x daily

(Recommended for 3-6 months after Phase 2)

Post-Phase 3 Maintenance Dosage: 1x Mara-Gen daily until full 5 years of no recurrence.

Notes:

For patients who cannot drink much, the water amount may be reduced to 350ml. The water is for AIC absorption, not AIC-SG. AIC-SG alone can be taken with 100-200ml of water for full absorption. Please take AIC and AIC-SG on an empty stomach not to compete with other glucose. We recommend minimum of 3 months of AIC-SG. In rare cases, cancer patients may need to supplement with potassium as high calcium ionic intake in normal cells can lead to a low intracellular potassium level.

TREATMENT GUIDELINES

Combination Therapy Dosing:

5ml of AIC and 5ml of AIC-SG into 500ml of water for three doses a day on an empty stomach for 30 minutes. Please do not mix it with food or other supplements. AIC-SG is best taken on an empty stomach when hunger is experienced for the best absorption by tumor cells.

For better dosing compliance, the amount of water for AIC + AIC-SG combination therapy could be reduced to 350 ml and slowly raised back to 500ml if compliance is an issue.

AIC-SG Therapy Dosing:

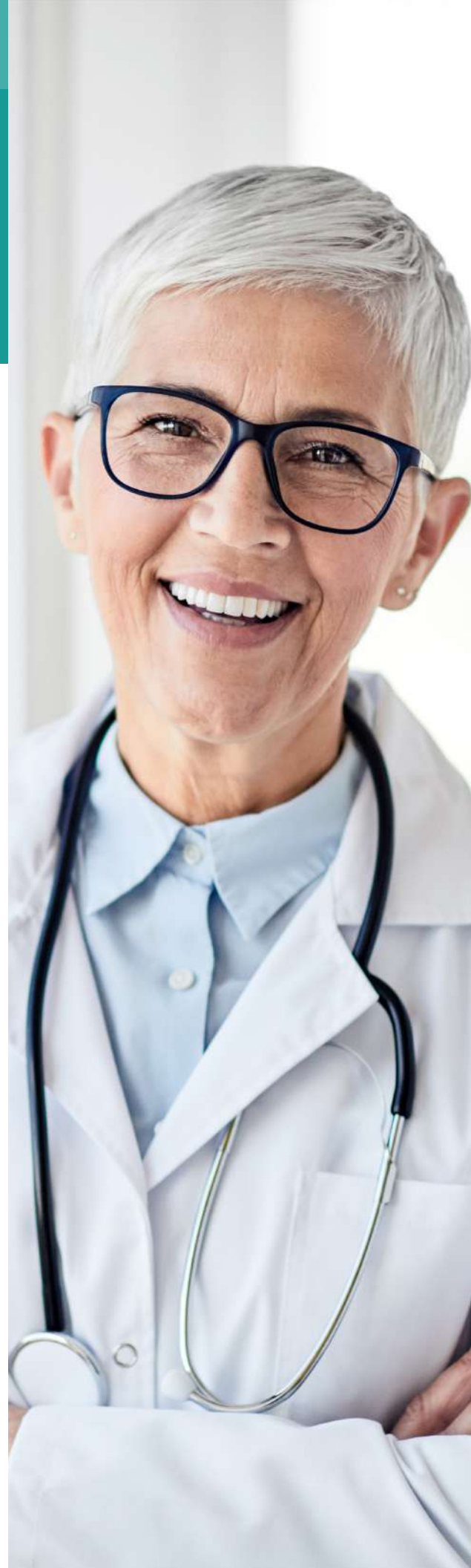
If administering just AIC-SG alone, lesser volume of water is needed. Recommended amount of water is 100-200ml.

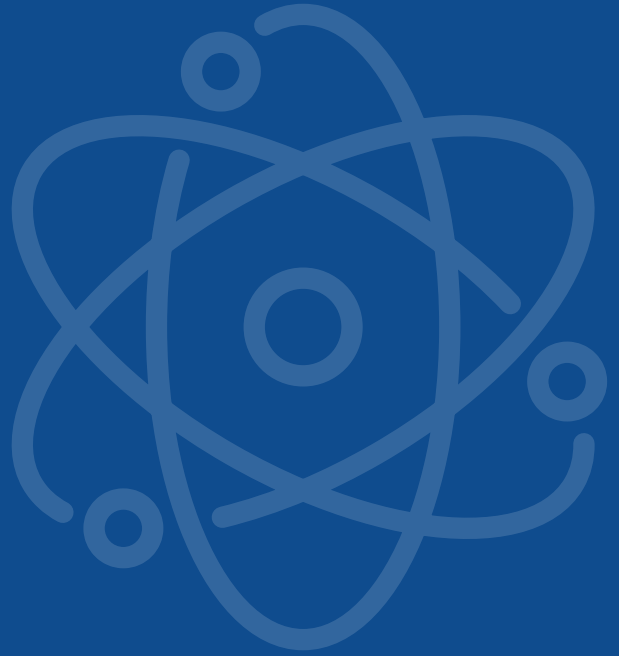
Duration of the Therapy:

Recommended period of treatment is minimum three months to turn around the disease up to stage 3. Advanced stage may require more time and higher dosing. Continue the therapy until symptoms of the disease disappear in scans and lab tests. When symptoms of the disease recede, continue only with three doses of AIC for 3-6 months and drop down to two doses for another 3-6 months. Going forward, stay with one dose of AIC as a preventive measure. Routine screening with oncologists recommended.

Contraindications:

No Contraindications with Conventional Cancer Treatments: the therapy can be taken safely while undergoing conventional treatments. The therapy could also help mitigate the side effects of conventional treatments. Because of effective response of the therapy, we recommend trying out the therapy for a month first before committing to conventional aggressive treatments that comes with physical burdens.





CLINICAL CASES

- All chronic degenerative diseases feature increased extracellular and intracellular levels of calcium.
- The increased intracellular calcium, which always results in increased intracellular oxidative stress
- Chronic loss of calcium from bones continually feeds the excess presence of calcium in the body.

- *DEATH BY CALCIUM* by Dr. Thomas Levy



WHAT DO OUR PATIENTS SAY?



AIC therapy is helping my daughter's **Crohn's disease**!

JOANNE B. - Age 66, Female, Canada



My **multiple myeloma** is no more. I am cancer free.

MOLLY HOLT - Age 80, Female, South Korea



I recovered from deadly heart condition, **mitral valve prolapse**! AIC therapy gave me new life!

E. BARAGOOSH, - Age 59, Male, Canada



My **blood sugar level** dropped by 50 points!

Y. S. KWAK - Age 69, Female, NYC



Arthritis pain and swelling is gone! Amazing!

M. KIM - Age 53, Female



My bone density is normal. No more **osteoporosis**!

M. S. PARK - Age 55, Male

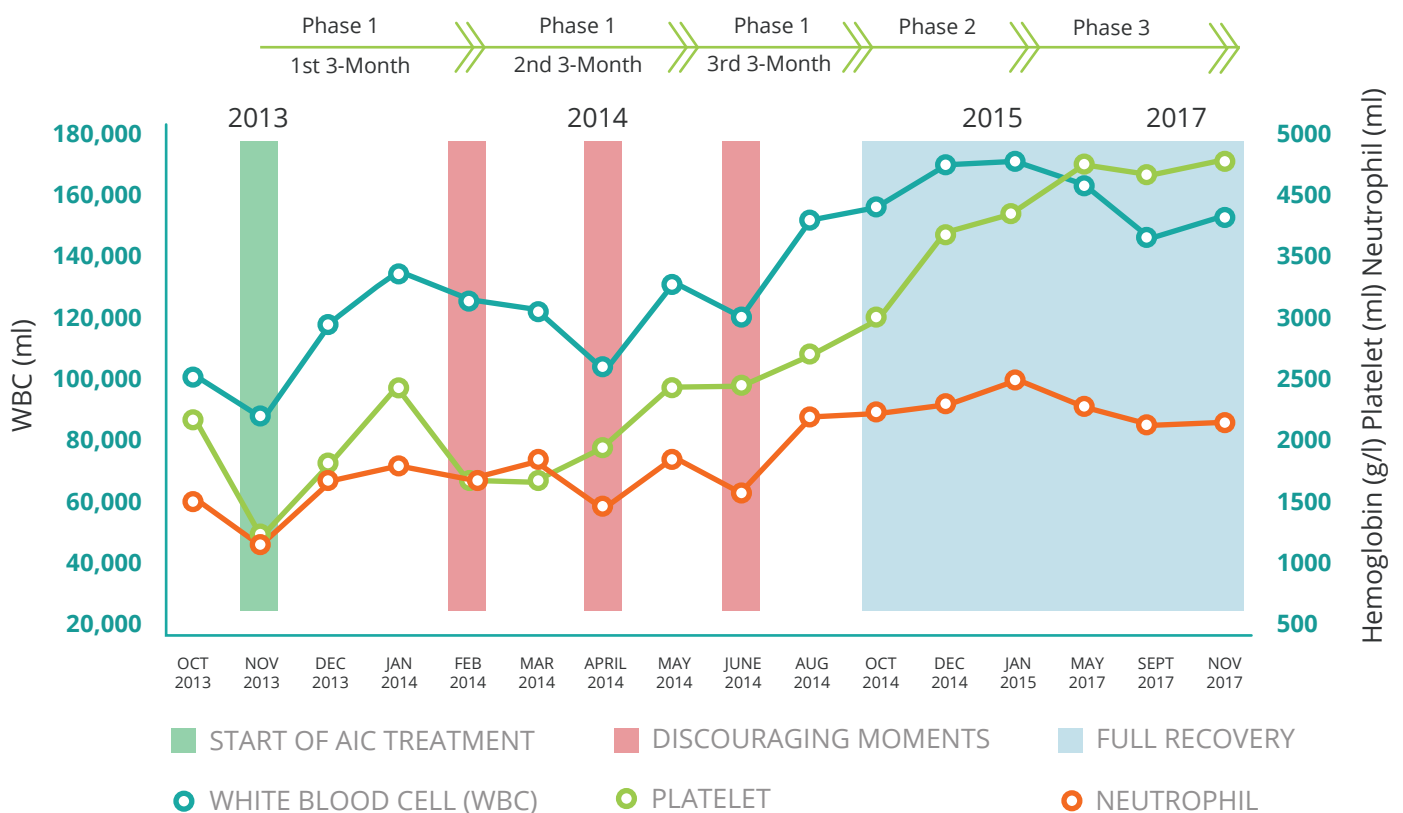
MULTIPLE MYELOMA CASE STUDY

In the case of an 80-year old lady, Molly H., with last stage multiple myeloma, the road to her full recovery had mixed signals.

In the first 3-month of AIC therapy, there was a clear sign that the therapy was working, followed by a dip that seems to indicate that it was not. Because there were some good signs, the second 3-month of treatment were given, which also had ups and downs. However, during the second 3-month period of treatment, there were more positive effects, including her general feeling of wellbeing.

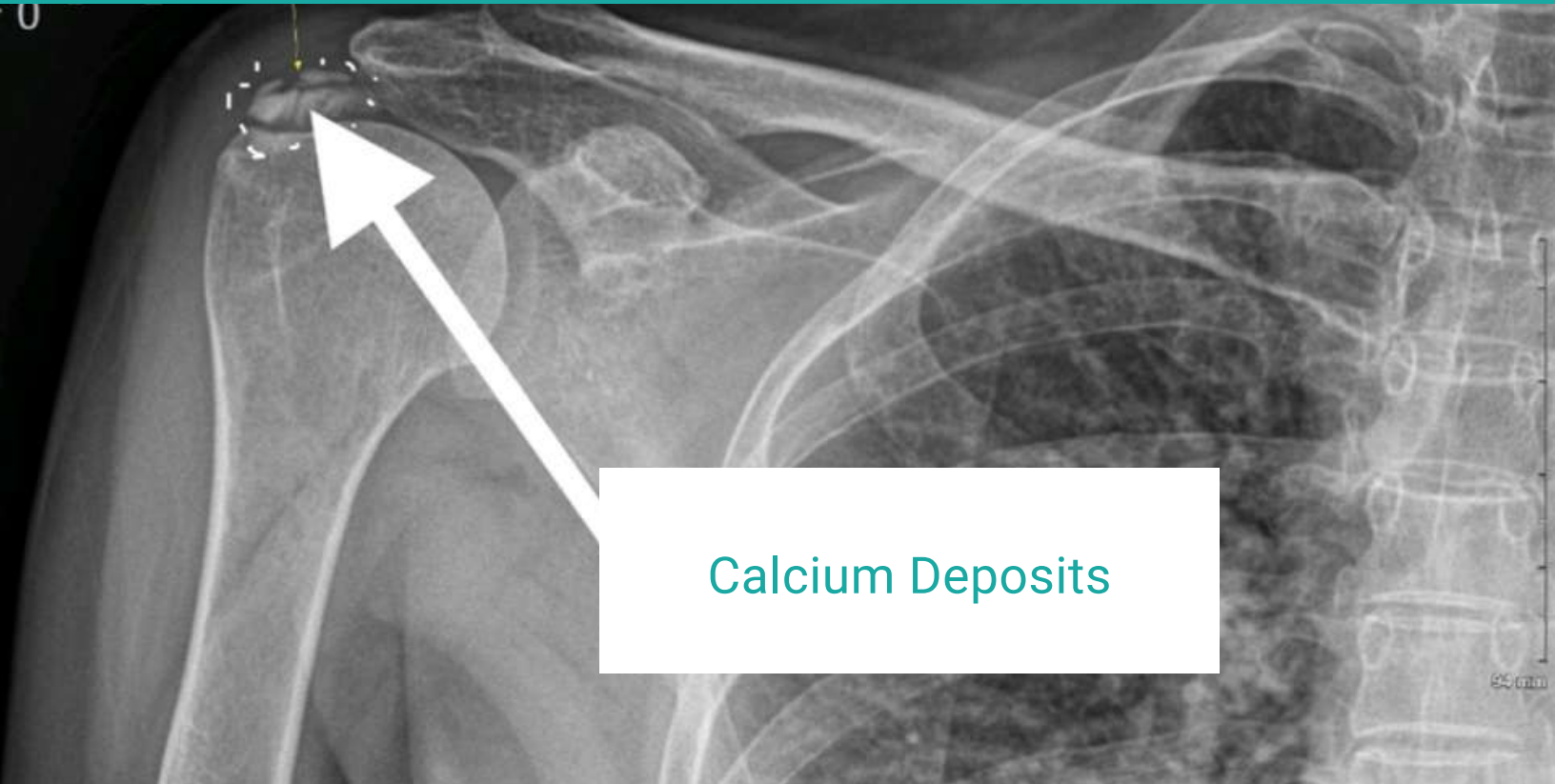
When Molly had committed to the third 3-month of treatment, her lab results and health steadily climbed up to full cancer remission. Her bone density returned to normal (-0.9) from osteoporosis (-3.4), and the kidney function bounced back up to GFR 62 from GFR 12. Her rib fracture healed quickly and thrombosis resolved as well.

Multiple Myeloma Case (80 F)

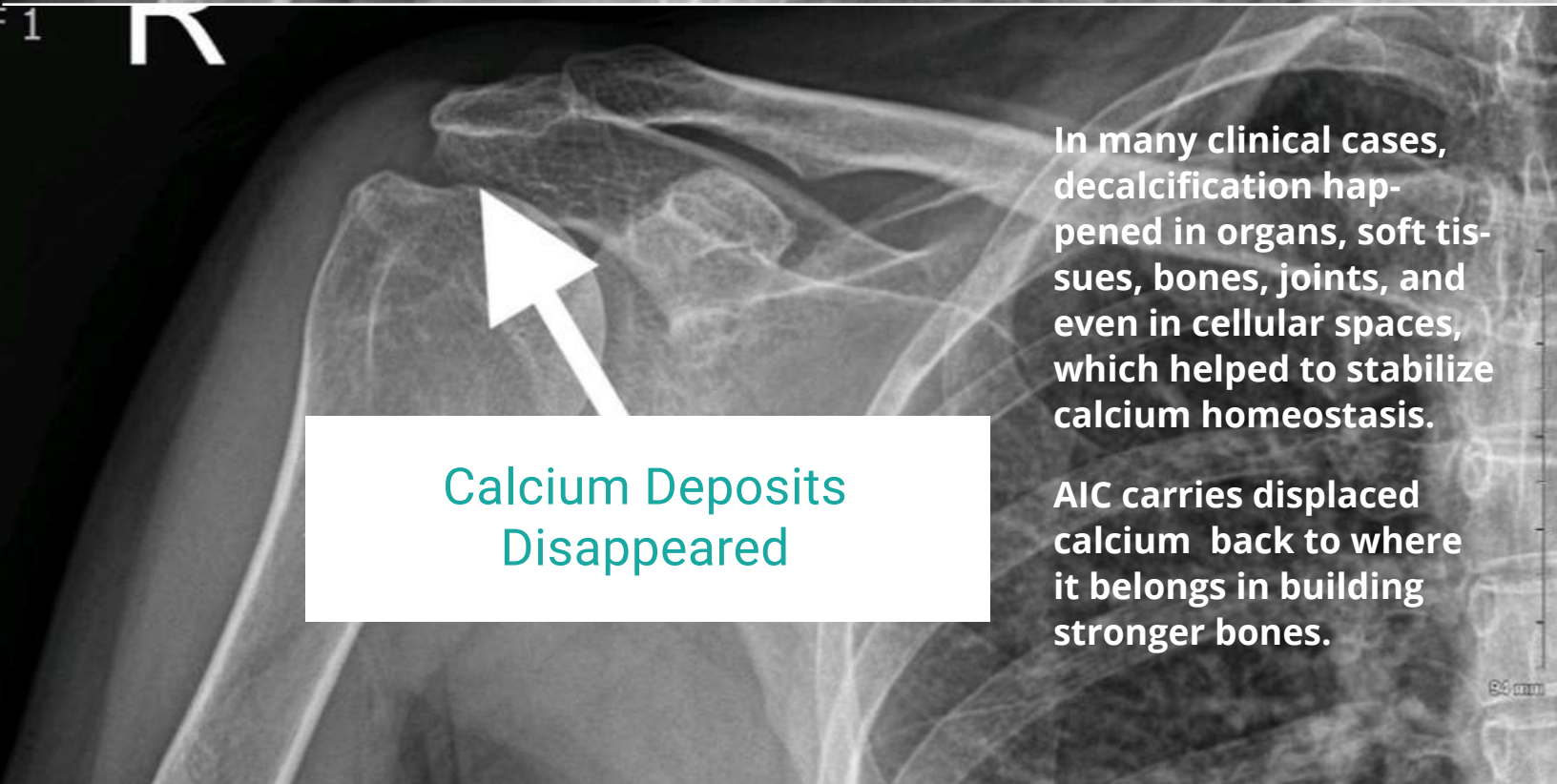


DE-CALCIFICATION EFFECTS OF AIC

(Calcific Tendonitis Case)



Calcium Deposits



Calcium Deposits
Disappeared

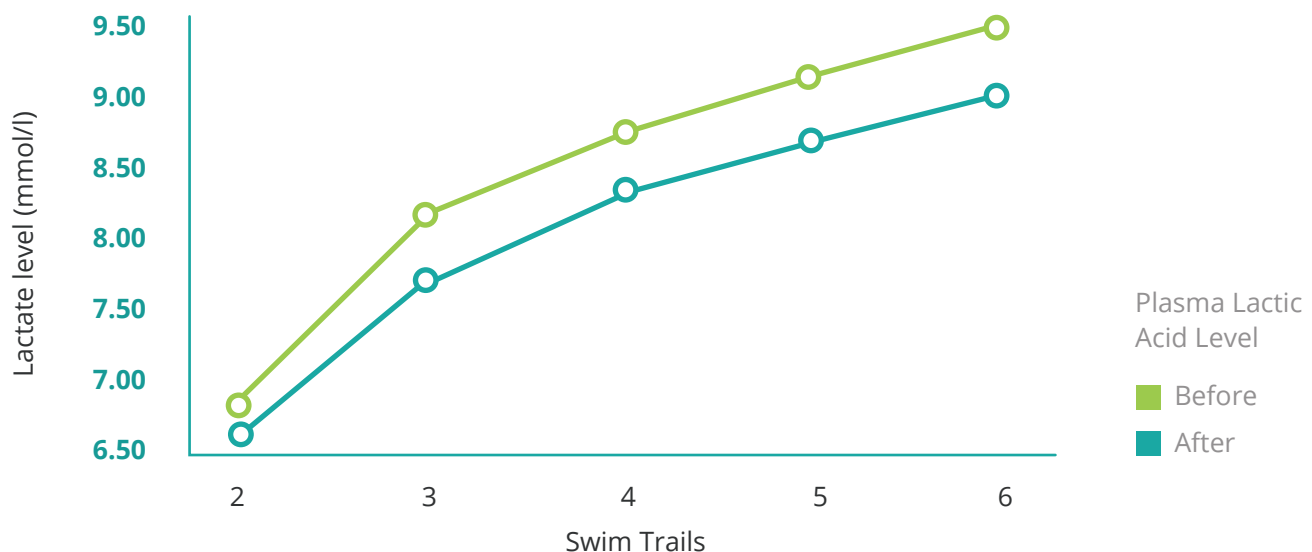
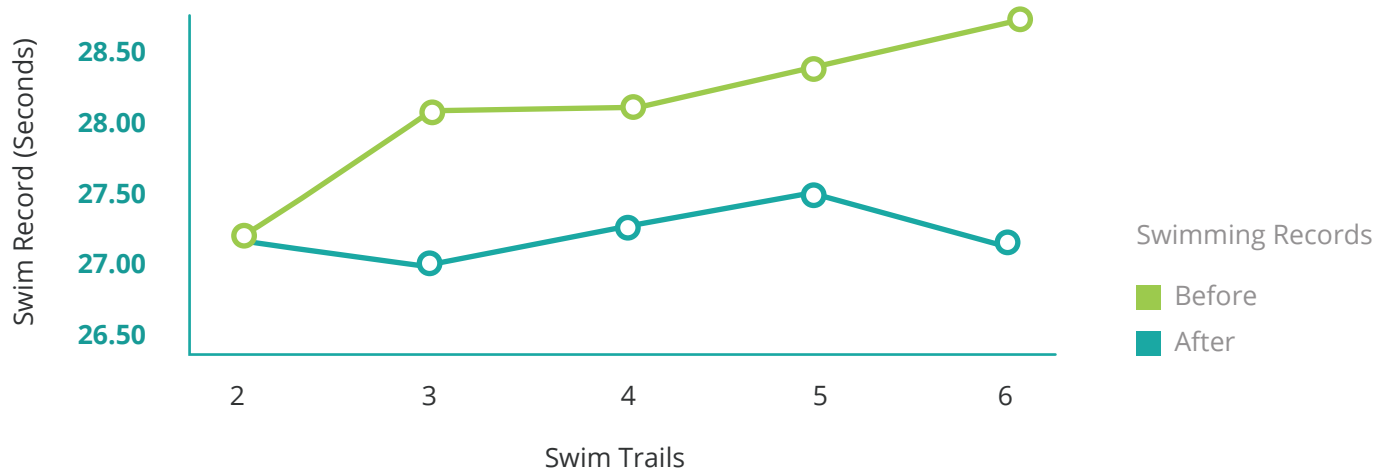
In many clinical cases, decalcification happened in organs, soft tissues, bones, joints, and even in cellular spaces, which helped to stabilize calcium homeostasis.

AIC carries displaced calcium back to where it belongs in building stronger bones.

LACTIC ACID REDUCTION CLINICAL CASES

Lactic acid, which is the byproduct of carcinoma cancer cells, is known to inhibit the immune functions around cancer tumors. Neutralizing the lactic acid and bringing pH back to a normal level is crucial in cancer treatment.

Twenty swimmers from Korea University swim team taking two doses a day for 14 days proved that AIC neutralizes lactic acid effectively. They all broke their records as a result.



ANIMAL TRIALS OSTEOPOROSIS



Osteoporosis animal trial proved to be very promising. In the bone break test, AIC built much stronger bones than other groups given regular calcium supplements. Also, rapid bone loss of osteoporosis-induced ovariectomized rats was fully recovered (100%) with AIC. The levels of osteocalcin, estradiol, and CTx normalized as well.

	Estradiol (pg/ml)	Osteocalcin (ng/mL)	C-terminal telopeptide (ng/ml)
Control	4.536±0.343 ^a	3.398±0.294 ^a	4.751±0.244 ^a
Osteoporosis	3.920±0.277 ^a	4.047±0.501 ^b	5.082±0.462 ^{ab}
Osteop.+ AIC	4.217±0.247 ^a	3.558±0.154 ^{ac}	4.360±0.495 ^a

BONE STRENGTH TRIAL		
	Dose Concentration	BMD
AIC Calcium	0.0001%	102.2
Regular Calcium	0.01%	96.2

(AIC shows 100x effectiveness in bone strengthening power. 1ppm daily for 9 weeks)

OSTEOPOROSIS RECOVERY TRIAL (BMD)		
Control	placebo	0.2276
OVX (osteoporosis)	placebo	0.1965
OVX + AIC	AIC	0.2276

(AIC restores bone density fully to normal level from osteoporosis)

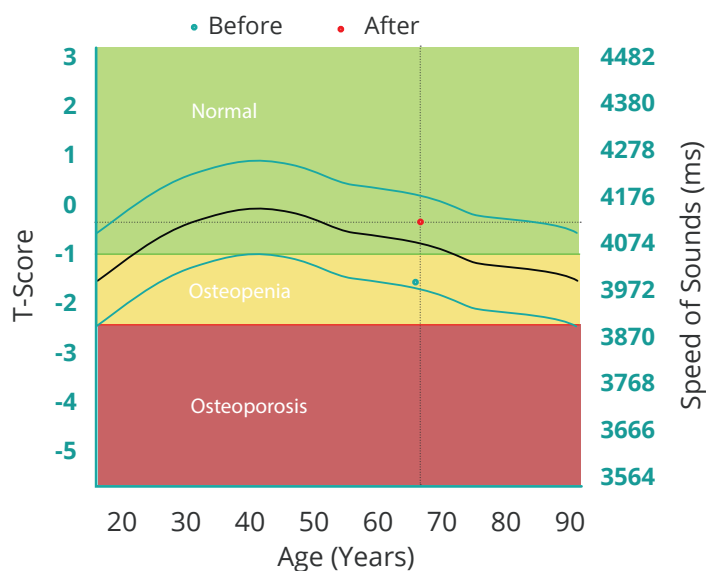
SAMPLE CLINICAL BONE DENSITY INCREASE



AGE: 66M

T-Score Increase: 1.4

From osteopenia to normal in 4 months.



04/22/2015 (After)*

Age: 67 | T- Score: -0.3 | Z-Score: 0.6 | SOS: 4107

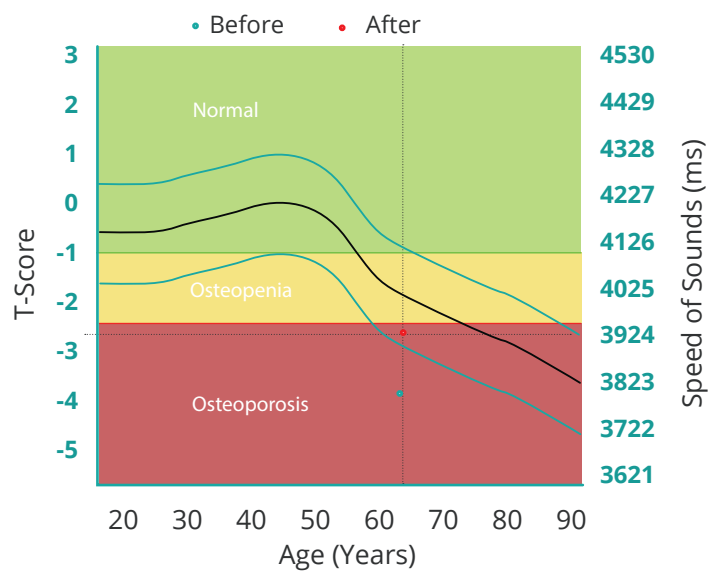
12/30/2014 (Before)

Age: 66 | T- Score: -1.7 | Z-Score: 0.8 | SOS: 3947

AGE: 62F

T-Score Increase: 1.2

From severe osteoporosis to osteopenia in 3 months.



04/22/2015 (After)

Age: 63 | T- Score: -2.7 | Z-Score: -0.7 | SOS: 3877

01/23/2015 (Before)

Age: 62 | T- Score: -3.9 | Z-Score: -1.9 | SOS: 3740

*CBHI utilized FDA approved ultrasound bone densitometer by BeamMed in measuring and comparing BMD data of more than a thousand patients. Over 90% of the patients experienced increased bone density.

FRACTURE HEALING EFFECTS OF AIC

Steroid Induced Osteoporosis, auto fracture (Male, 52, Indonesia)

Dosage : Mara-Gen 2x /day for first 2 months and then only 1x.
Able to walk normally again.



March 7, 2018



July 27, 2018



Dec 31, 2018

In the bone cancer case below, a young girl did not have to undergo amputation. AIC's powerful effects on bone modeling significantly affects bone marrow as well.



Female, Age : 10



After 1 month

DENTAL CAVITATION & IMPLANTS

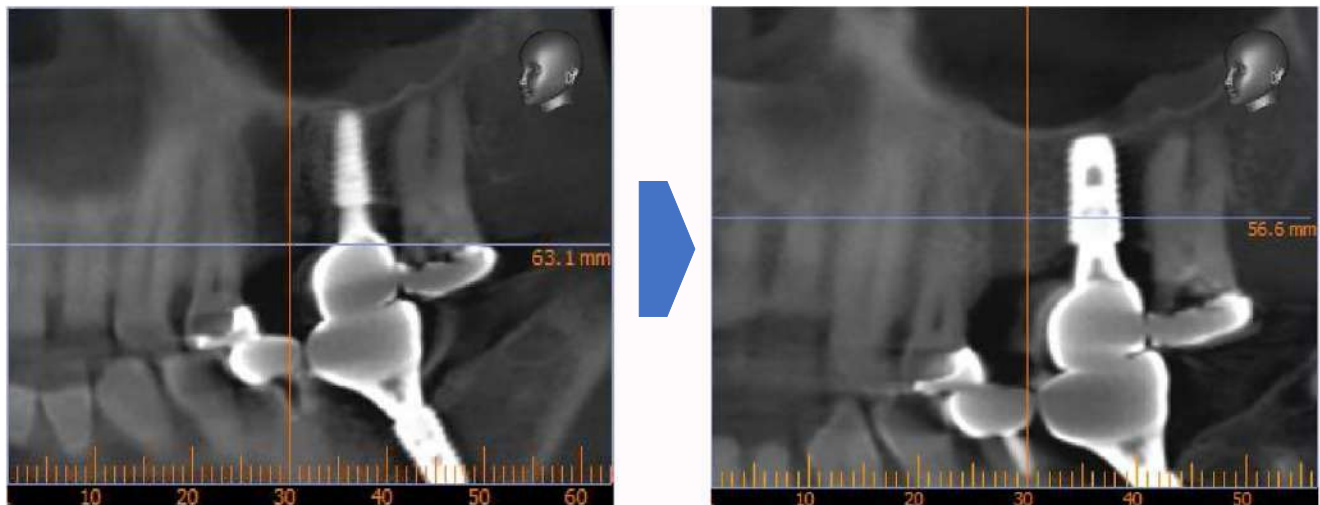
Infected tooth removal. (Male, 60, Canada)

Dosage : Mara-Gen 2x /day & Denti-gen 1x/day. Cavitation filled up without any bone grafts. His bone T-score rose as well.



Dental Implant now possible. (Male, 64, Canada)

Dosage : Mara-Gen 2x /day for 40 days. Implants became possible.



In dental applications, AIC effectively triggers hPDLs (human periodontal ligament cells) to build stronger alveolar bones on both mandible and maxilla. After cleaning, cavitations resulted from extracted teeth were filled up with natural bone adequately without bone grafts, and implants showed tighter osseointegration as well as stronger bony support. AIC also normalizes Ca^{2+} and phosphorus ion concentrations and ratios which significantly affects proliferation and differentiation of dental stem cells.

ATOPIC DERMATITIS

Atopic Dermatitis (Female, 41, Canada)

Dosage : Mara-Gen 3x /day.



Atopic Dermatitis Case

- Ms. H. T. Kim, Female, South Korea
- Suffered Eczema since childhood
- MaraGen 2x/day for one year.
- Experienced healing reactions for 3-6months but continued.



ECZEMA CASE



**Used steroid cream
without success
(Female, Singapore)**

Dosage:
Mara-Gen 3x /day.

PSORIASIS CASE

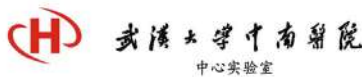


**Used steroid cream
without success
(Female, 25, Jakarta)**

Dosage:
Mara-Gen 3x /day.

COVID-19 CLINICAL STUDY

In March, 2020, Wuhan University Hospital put seven patients under AIC therapy as many patients were dying of the original strain of COVID virus. All the patients took Mara-Gen 3x daily for a month and recovered fully and got discharged.



新型冠状病毒治疗方案样品检测数据报告

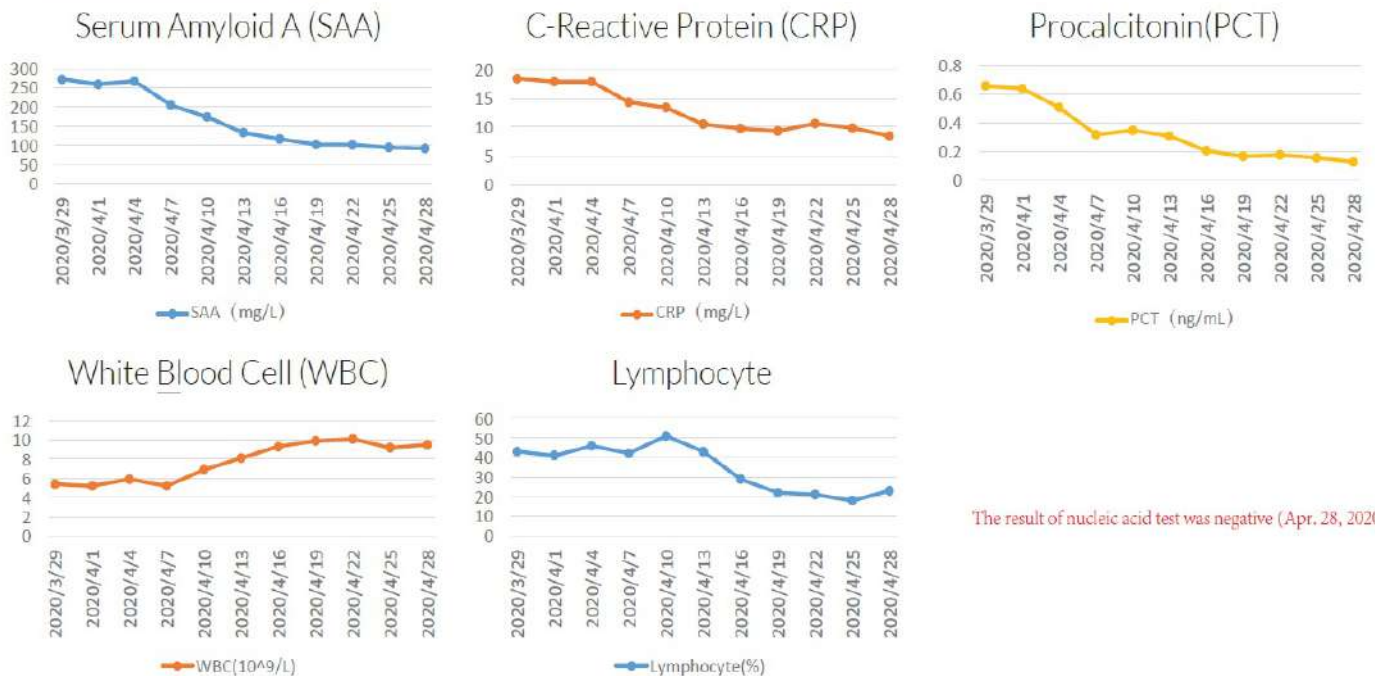
2020年7月25日



Sample Test Data Report For COVID-19 Treatments

July 25, 2020

Zhengrong Pang - COVID 19 active. Age 27, Male.
The result of nucleic acid test was positive and diagnosed with COVID-19 on Mar. 28, 2020
By Wuhan Univ. Zhongnan Hospital. Central Lab
Medicine: SAC (CBHI, Made in Canada)



The result of nucleic acid test was negative (Apr. 28, 2020)

DEMENTIA CASE WITH ARTHRITIS

Athletic male with dementia (Male, 74, Singapore)

Dosage : Mara-Gen 3x /day

Diagnosis:

- Diagnosis: Early Dementia, June 2017
- Eczema, Multiple Allergies

Medical/Personal Timeline:

- 1999, hernia and slipped disc. Prior to this, the patient used to run 45 km a week regularly until his slip disc gave problems. He switched to walking 40 km a week until his knees gave problems. Taken part in marathons.
- 2017, Dementia grew significantly worse as he was not able to find ways to find home.
- **June 2019, AIC Therapy started**
- March 2020, His abilities to remember conversations and names returned. Less of misplacing of items and was able to remember ways to find home.
- May 2020, Noted reduction in weight. Waistline reduced by 10 cm. He was able to use his knees better.
- June 2020, His scheduled knee caps operations cancelled.
- After July 2021, He could walked more than 10 km without stopping.
- August 2021, He is back to walking 6 km daily.
- September 2021, He walks 40 km weekly.

DEMENTIA CASE

Diagnosis: Dementia with cognitive decline (Male, 83, Singapore)

Dosage : Mara-Gen 3x /day

Medical/Personal Timeline:

- Constantly calling the 60 year old son not to leave him at home during work. Kept questioning the son why he left him at home. He felt abandoned and threw tantrums.
- **Started AIC Ionic Calcium Therapy in July 2019**
- In December 2019 the frequency of the calls to the son reduced
- In June 2020, he was able to understand that the son went to work and could stay home by himself and even started having his own activities.

EPILEPSY CASE

Diagnosis: Autism diagnosed at 2.5 years old. (Female, 16, Singapore)
Developed Grand Mal Seizure in 2014

Dosage : Mara-Gen 3x /day

Medical/Personal Timeline:

- Received stem cell treatment in Germany in 2012 when she was 7 years old. 4 treatments in 2 years. Seizures developed after the treatment.
- TDCS (Transcranial Direct Current Stimulations) 4 sessions in 2018. Frequency of seizures doubled after the TDCS treatment.
- **July 2019 - Started AIC Therapy**
- December 2000 - 28 days without seizure
- July 2020 - 35 days without seizure
- Jan 2021 - 41 days without seizure
- March 2021 - 54 days without seizure
- July 2021 - 66 days without seizure
- January 2022 - 86 days without seizure

AUTISM CASE

Diagnosis: Autism Spectrum Disorder (Female, 16, Singapore)

Dosage : Mara-Gen 3x /day

Medical/Personal Timeline:

- Diagnosed with mild autism at 24 months old
- Attended special schools. Had issues with behaviour and sensory integration
- July 2019 - She came in for consultation. She had major breakdowns and refused to go to school and locked herself in her room.
- **August 2019 - Started AIC Therapy**
- October 2019 - Was able to study again at home with home teacher.
- November 2019 - Attention span increase and could have 4 lessons in a day
- January 2020 - Willing to do subjects that she didn't like before (Economics)
- March 2020 - Wrote a letter to her favourite YouTuber (Her first letter written)
- July 2020 - Started having longer conversations with her teachers
- September 2020 - Was able to attend a social event without tantrums
- February 2021 - Attended online schools
- In the year of 2021 - Notable increase in cognitive and social abilities
- March 2022 - Taking part in Secondary School Examination (normal stream)

"Calcium signaling is involved in many essential cellular functions that span multiple organelles, tissues, and physiological systems. Given its ubiquitous role, it is predictable that a deregulation along any point in the calcium signaling pathway may be capable of disrupting neurological function, which has already been demonstrated by a number of 'calciumopathies' – a diverse group of diseases caused by a disruption of calcium homeostasis."



CERVICAL CANCER CASE STUDY

Diagnosed with Cervical Cancer (2012) Joanna M. (Female, 39, USA)
Dosage: Mara-Gen 3-4x Daily

2012 :	Diagnosed with cervical cancer. Underwent radical trachelectomy
April 2016 :	Recurrence of cancer.
Nov 2016 :	Chemotherapy
Jan 2017 :	Tumor grows back
Feb-May 2017 :	Chemo and Radiation Therapy
May 2017 :	Ongoing presence of cancer discovered
May 2017 :	Starts AIC Therapy (AIC 3-4x /day)
Sept 2017 :	Declared cancer free after TRU CUT Biopsy
Sept 2017 :	Stops AIC Therapy prematurely
Feb 2018 :	Recurrence of Cancer
May-Sept 2019:	Cryoablation Ethanol Ablation in Guangzhou, China.
Oct 2019:	Tumor size decreased but cancer still present.
Nov 2019 :	Started AIC Therapy again (AIC 4x/day)
Sept 2020 :	Declared cancer free (Again)
Aug 2021 :	Negative after PET scan
2021- 2023 :	Taking 1x Mara-Gen daily. Cancer free.



THE
STORY
2021

2022

2023

In 2012, I was diagnosed with cervical cancer. My husband and I wanted to have kids then, so I didn't want to undergo hysterectomy, and chemo and radiation was also out of the question because I was very afraid of the effects on my body. I asked my oncologist what my options were, and she said they can try to remove just part of my cervix which is what I underwent. The procedure is called radical trachelectomy (removal of the cervix) and I was clear.

Unfortunately, in April 2016, the cancer came back. This time, after trying my hardest to avoid the chemo and radiation route again, I had to succumb to it. I was in a lot of pain, so at point I didn't have much of a choice. I started chemotherapy in September 2016 and finished in November. The plan was to shrink the tumor and have it removed. However, no oncologist wanted to touch me. By late January of 2017, the tumor grew again, and I was in pain again. I started my second round of chemo in February, this time with radiation therapy. I finished treatment in May 2017. The tumor shrank again but there was still cancer.

My oncologists didn't know what to do with me after because apparently, they've given me the maximum chemo dose a person's body can take so they said they'll just monitor me closely. In the meantime, my dad was introduced to AIC and he told me to give it a shot. I started taking in May. **I was taking it 3x a day everyday for 4 straight months. In September 2017, I had a biopsy done and my oncologist said they couldn't find any more cancer.** It was a miracle! AIC is a bit pricey so when I found out that there was no more cancer, I decided to stop taking it.

In late January of 2018, I started feeling pain again in my lower back and left abdomen. In February, I went to see my oncologist and found out that there was cancer again. I was devastated. They told me if I wanted to undergo chemotherapy again—try out a different drug or immunotherapy. I suffered so much from the previous treatments, so I didn't want to go through the whole thing again. I decided to go the alternative route (again).

In May 2019, we ended up going to a cancer hospital in China. I had cryoablation (freezing the tumor), ethanol ablation (injecting ethanol directly to the tumor) localized (administered directly to the tumor as well) and targeted chemotherapy. After 4 months of treatment, still there was cancer.

We flew back to the US in October 2019 and were supposed to go back to China in January, but we didn't (due to financial reasons). I started taking AIC again in November 2019 because when I saw my oncologist again here in Vegas, she didn't have anything new to offer me except more chemo and/or immunotherapy. Something that I didn't want to do yet another time. She said she'll just closely monitor me then and hope for the best. I returned to taking AIC 4x a day and this time I was instructed not to stop midway, and that's what I did. **Last month, I had my checkup done and surprisingly, my oncologist said that she could not find any mass AT ALL. She couldn't see or feel anything. She was shocked! I wasn't. I knew it was AIC.**

LYTIC LESIONS IN BREAST CANCER

Stage 4 Breast Cancer with Lytic Lesions (Female, 80, USA)

Dosage : Mara-Gen 4x /day

Diagnosis: July 2018

Triple negative breast carcinoma (TNBC-Triple Negative, right breast)

Extensive metastasis to bone (Multiple spinal axial lesions, most prominent in T9 and T12, multiple subacute to chronic fractures in ribcage)

Medical/Personal Timeline:

March 2017: CT, Bone Scan, PET, Bone marrow biopsy, mammogram conducted without cancer diagnosis

Feb 2017: Thyroidectomy

May 2017: Hysterectomy

Sept 2017: Loss of husband to cancer

Jul 2018: Diagnosed with carcinoma of right breast with mets to bones (T9,T12 and ribcage)

July 2018: The patient received alternative treatments at Hope4Cancer clinic with AIC therapy.

AIC Therapy started for bone lytic lesions.

CT ABDOMINAL/PELVIS w/o CONTRAST



July 6, 2018

CT scan at dorsal spine (T10) showing lytic lesions in vertebral body



Nov 26, 2018

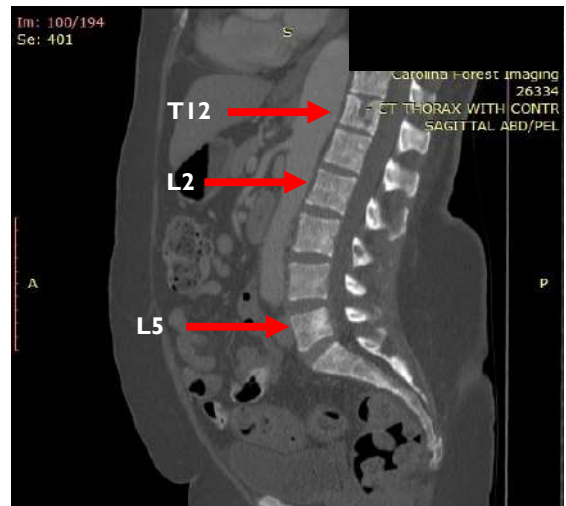
Control CT showing reduction in size of lytic lesions

CT ABDOMEN/PELVIS



Jul 6, 2018

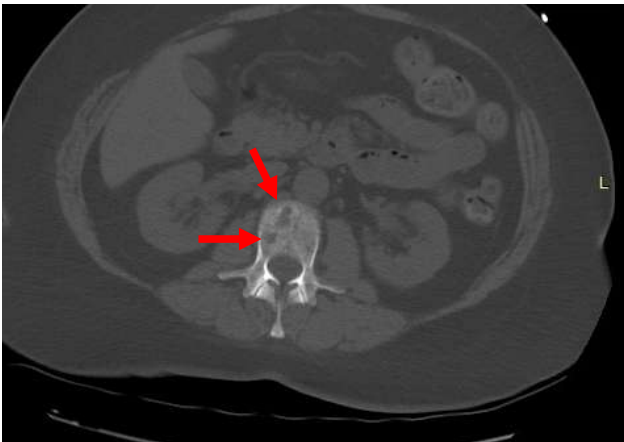
Sagittal reformation of lumbar spine
Lytic lesions in vertebral bodies T12, L2, and L5



June 4, 2019

Lytic lesion in T12 is smaller. No evidence of lesion in L2. Positive response in L5.

CT ABDOMEN/PELVIS: AXIAL IMAGE THROUGH L2



Jul 6, 2018

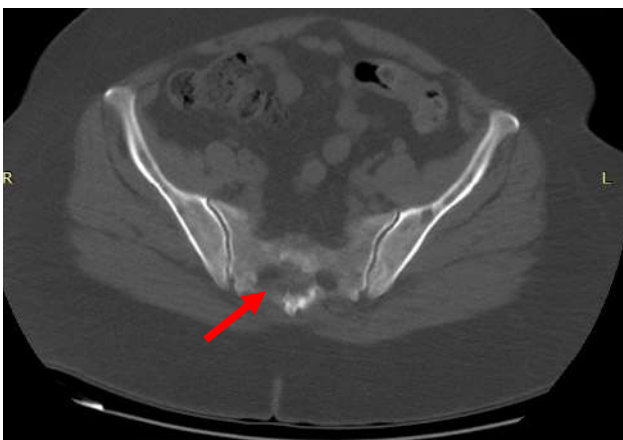
Two lytic lesions in vertebral body



June 4, 2019

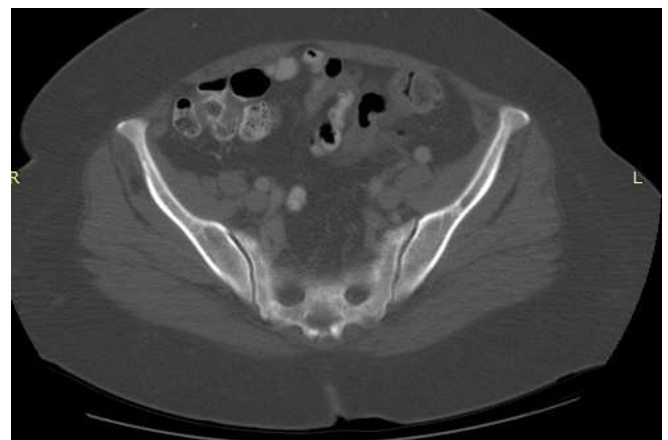
No evidence of lesions

CT ABDOMEN/PELVIS: AXIAL IMAGE IN PELVIS



Jul 6, 2018

Lytic lesion in sacrum



June 4, 2019

No evidence of lesion



OVARIAN CANCER AIC IV THERAPY

Stage 4 Ovarian Cancer Diagnosed in 2019 (Ms. Jin, Female, 59, China)
Dosage : Mara-Gen IV 10ml 2x /day

Medical/Personal Timeline:

December, 2018

Oophorectomy performed

October, 2019

Chemo and radiation treatment stopped as Ca125 rose above 10,000 and cancer continued to spread all over the body.

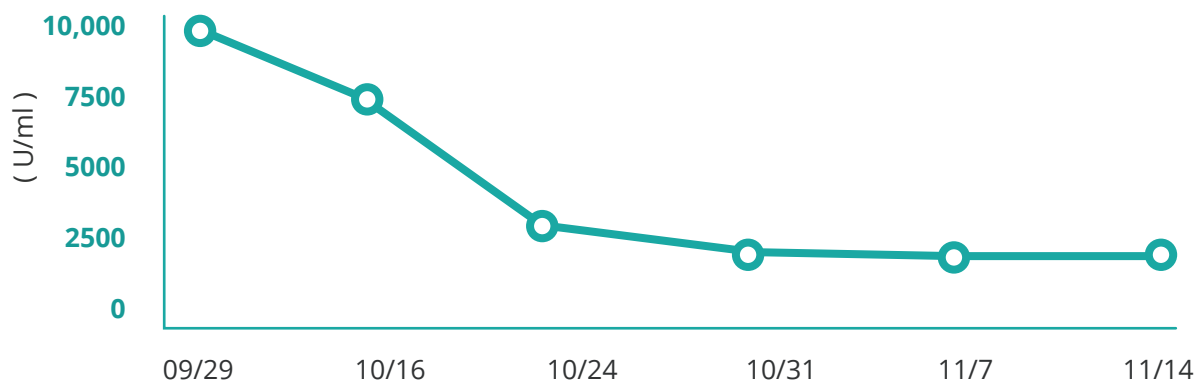
October, 2019

Mara-Gen IV treatment started with 10ml of Mara-Gen in 250ml 0.9 saline solution 2x/day. No other therapies administered.

Clinical improvements experienced within 2 months of AIC therapy:

- Stomach much more comfortable with increased appetite and more nutritional absorption.
- No longer needs sleeping pills to fall asleep
- Every morning trip to bathroom now possible. Difficulty of urination and pooping alleviated.
- Emotionally brighter and her face looks lively. Started gardening and house works again.

CA125 Measurements under Mara-Gen IV:



PROSTATE CANCER & LIVER METASTASIS

Stage 4 Prostate Cancer with Liver Metastasis Diagnosed in 2018

(Mr. James C., Male, 58, Canada)

Dosage : Mara-Gen 3-4x /day

Medical/Personal Timeline:

- May 2018, CT Abdomen and Pelvis Enhanced showed innumerable low density lesions throughout the liver the largest within segment 4a measuring 3.3cm in maximum diameter.
- June 2018, Needle core biopsy shows liver infiltrated by an metastatic adenocarcinoma of prostate origin.
- June 2018, CT Chest shows several enlarged left supraclavicular lymph nodes, with the largest measuring 1.7cm in size.
- June 2018, Bone scan shows multiple bone metastasis

AIC Therapy started in July, 2018

- Oct 8, 2018, CT Abdomen shows: The segment 4a lesion has decreased to 1.6 cm from 3.3 cm. Most of the other lesions have also decreased in size. No definite new lesions in the liver. The prostate gland has decreased in size with no new focal lesions.
- Multiple lower retroperitoneal and iliac chain lymph nodes have decreased in size. A lymph node just inferior to the aortic bifurcation has decreased to 10 mm from 23 mm. No definite new lymphadenopathy.

PSA Marker Measurements Before and After AIC Therapy

AIC Therapy Started						
	6/1	6/18	7/18	8/14	9/13	10/9
PSA	26.0 µg/L	46.0 µg/L	6.27 µg/L	2.62 µg/L	1.70 µg/L	0.99 µg/L

(Normal Reference Range of PSA: 0-4.50 µg/L)

- No concurrent conventional treatments done with AIC therapy
- AIC therapy is given from the first week of July, 2018
- Dosage: Mara-Gen 3-4 / day

AIC-SG THERAPY IN BREAST CANCER

Stage 4 Breast Cancer with Bone Metastasis (Male, Age 67, Canada)

Dosage : Mara-Gen 3x /day, AIC-SG 3x/day

Diagnosis: (Feb. 2019)

Breast cancer stage 4. (HER2+)
Metastasis on bones and several locations
Couldn't get out of bed due to back pain (bone metastasis)
Given a few months to live.

Pre-trial Treatments:

Surgery (Radical mastectomy, removing 600g of tumor/flesh)
Chemotherapy, radiotherapy,
2nd chemo etc. (2020-present)

AIC+AIC-SG Treatment (parallel with chemo)

Started AIC therapy in April, 2022 and conditions improved faster.
AIC-SG is added to AIC @ 3x a day since Oct. 5, 2022.
Back pain is reduced to 10-20% by Oct 11, 2022
Tumor marker CA15-3 improved rapidly.



Feb

May

August

Mr. Peter Jung, Age 67, Male, BC, Canada
Metastatic Breast Cancer: HER2 Positive
Surgery, Radiation, Chemo (ADC T-DXd)

AIC-SG THERAPY IN RECTAL CANCER

Stage 3 Rectal Cancer (Male, Age 60, USA)

Dosage : Mara-Gen 3x /day, AIC-SG 3x/day

Diagnosis: (Oct. 2022)

Rectal Cancer Stage 3 diagnosed Oct. 4, 2022

Patient had colonoscopy (10/3/22) showing ulcerated and fungating mass 8cm from anus c/w moderately differentiated adenocarcinoma, MMR intact.

Pre-trial Treatments: Radiation ended on Dec. 2, 2022

AIC+AIC-SG Treatment (parallel with chemo)

Started AIC + AIC-SG Combination Therapy on Dec.14, 2022

4 rounds of chemo started on Dec, 2022 - Feb, 2023.

On February 10, three CT scans showed no evidence of the disease.

On March 1, the surgeon found no visible tumor on the site.

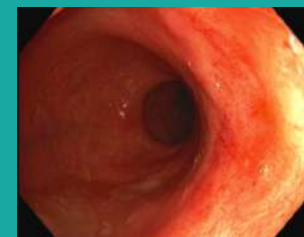
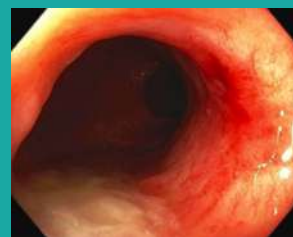
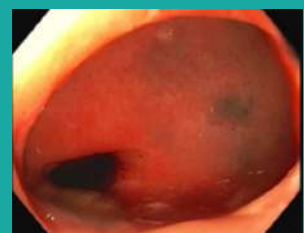
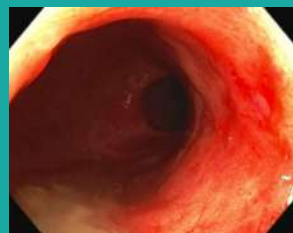
On March 15, MRI scan found no definitive evidence of residual disease.

On April 3, ctDNA Liquid Biopsy indicated negative.

On May 9, CT shows no evidence of acute pathology or metastatic disease.

On May 19, Biopsy highlights benign architecture. Immunohistochemistry for CMV is negative. Endosonography shows benign scars only.

On August 8, MRI shows no MR evidence of a discrete rectal mass and no evidence of metastatic disease.



Before (Oct 3, 2022)

After (May 15, 2023)

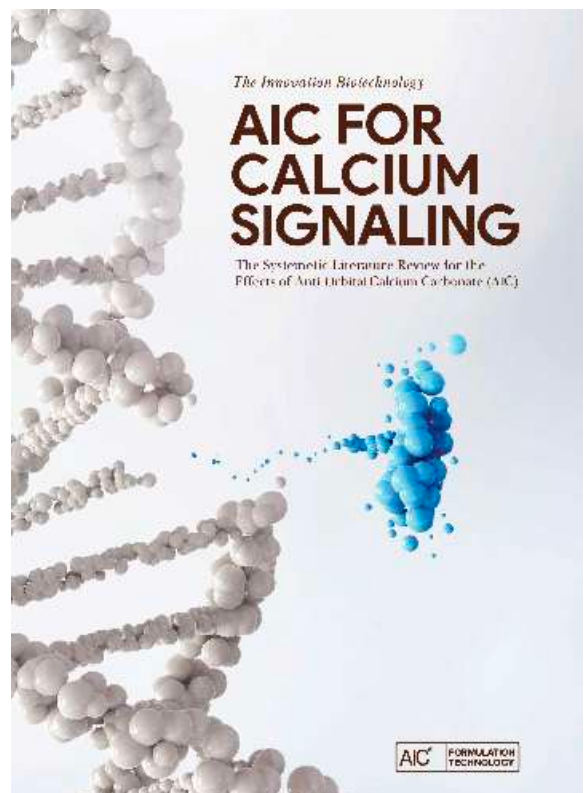


FOR MORE TECHNICAL EXPLANATIONS

FIND IT ON AMAZON,
or visit
AICTHERAPY.COM

"CBHI believes that if the healthy calcium level within all of our cells is well maintained, we will be free from hosts of debilitating diseases. I hope AIC calcium brings renewed hope for many who are suffering and would like to thank all the members of the CBHI Canada research team who contributed to the publishing of this book."

Paul K. Lee, Ph. D.





AIC THERAPY PRODUCT LINE UP

NPN 80043292 - CANADA



MARA-GEN VS MEGA-GEN

The information present in this section helps with understanding the differences in AIC content in the products. By comparing Mara-Gen which is used for treatment purposes and Mega-Gen which is used for maintenance, we gain a deeper understanding of the efficacy of different strength AIC products.

Absorption Rate & Effectiveness

Mara-Gen has 99% AIC purity and has 8 mg of AIC in 5 ml AIC solution
Mega-Gen has 90% AIC purity and has 5.0 mg of AIC in 5 ml AIC solution

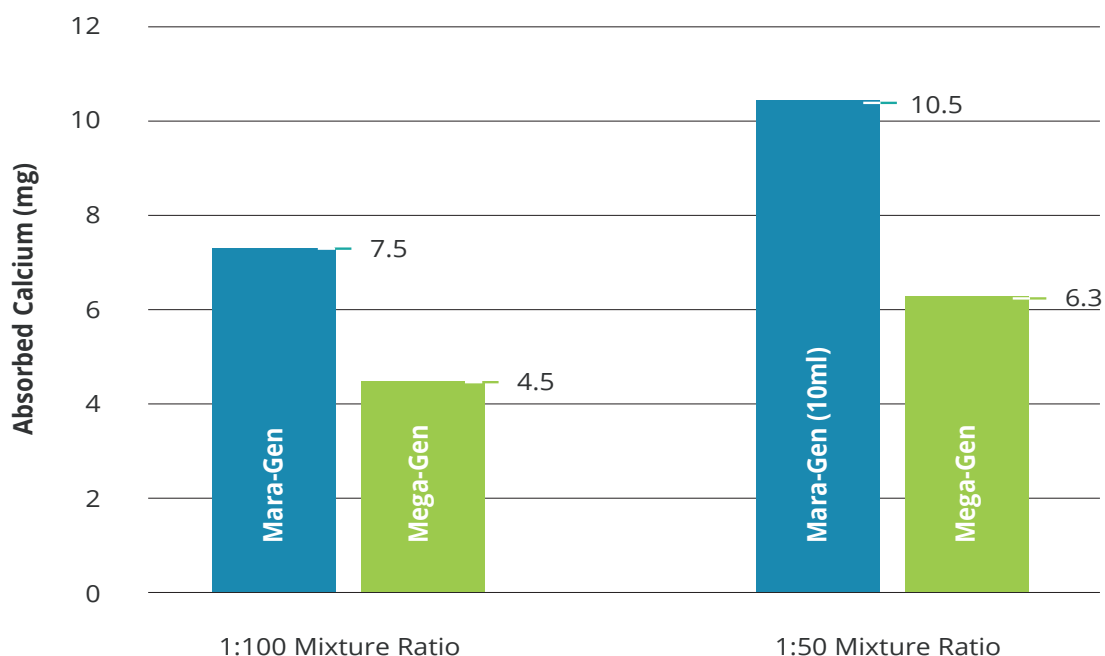
Example of oral intake of AIC

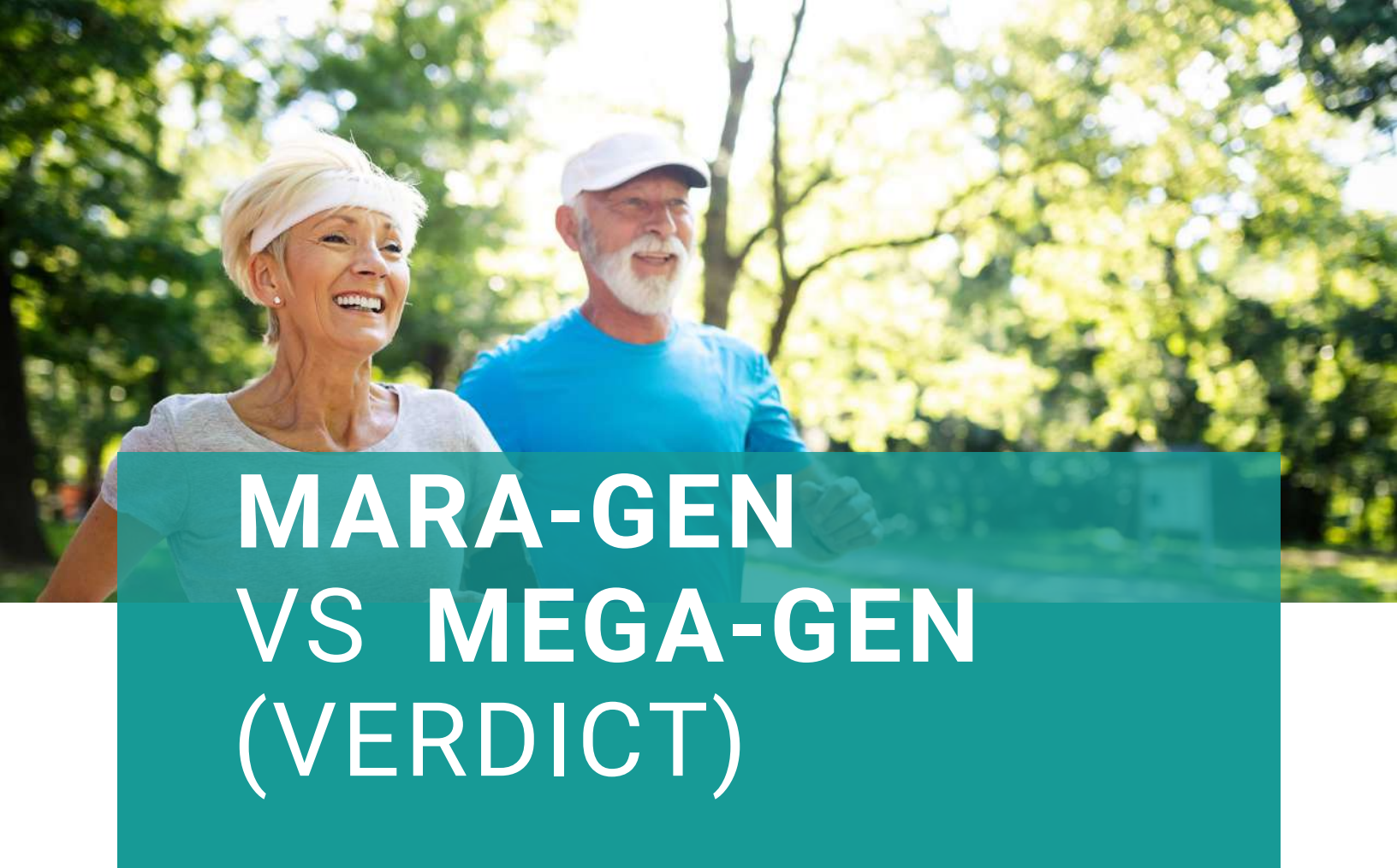
For example, 10 ml AIC in 500ml of water has a lower absorption rate, about 70% compared to 5 ml of AIC in 500 ml of water.

- Mara-Gen has 7.5 mg AIC absorbed at 5 ml in 500ml
- Mara-Gen has 10.5 mg AIC absorbed at 10 ml in 500ml
- Mega-Gen has 4.5 mg AIC absorbed at 5 ml in 500ml
- Mega-Gen has 6.3 mg AIC absorbed at 10 ml in 500ml

When we compare 10 ml Mega-Gen in 500ml of water to 5 ml Mara-Gen in 500 ml of water, Mara-Gen 5ml actually delivers a greater amount of AIC in our body than 10 ml of Mega-Gen, delivering more physiological effect even at half the amount of Mega-Gen.

Absorption Rate of Mara-Gen vs Mega-Gen:





MARA-GEN VS MEGA-GEN (VERDICT)

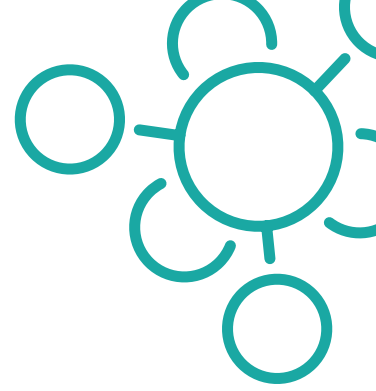
For most patients, Mara-Gen 5 ml (8 mg of AIC) will maximize the physiological ionic calcium benefits of the physiological moment, and the excess will be excreted, bringing down the serum ionic calcium level to the reference level within 4 hours. For these patients, repeating the dose every 3-4 hours will maximize the efficacy of AIC.

However, there are patients needing more ionic calcium where one dose of Mara-Gen (8 mg of AIC) is insufficient, and for such seriously ill patients, the following is recommended.

1. Mara-Gen IV which delivers 10-15 mg of AIC per session. (10 to 15 ml in 250 ml IV solution)
2. 10 ml of oral Mara-Gen mixed to 500ml of water, which delivers 11 mg of AIC at 30% loss of absorption rate but still provides a much stronger impact than 5 ml dosage, which provides 8 mg.

We have seen patients with huge ionic calcium needs recovering with multiple doses of standard 5 ml dose of Mara-Gen. However, 10 ml of oral Mara-Gen can give that extra push, although the treatment cost will be higher.

Mara-Gen is the default AIC product prescribed for treating illness. Mega-Gen is for healthy people to maintain calcium homeostasis for the prevention of diseases and the protection of overall health.



MARA-GEN

FORMULA FOR CANCER AND INCURABLE DISEASE

Mara-Gen is the most robust AIC-applied product, which aims to rebuild degenerated systemic health from the cellular level. By promoting decalcification from cellular to systemic level by stimulating osteoblasts, Mara-Gen helps to recover from over 150 degenerative diseases that are thought to be calcium-related. Many studies link cellular calcification to mitochondrial disease, metabolic syndrome, neurodegenerative diseases, and even cancer.

According to Mitochondrial Free Radical Theory, calcium ions accumulated in the walls of mitochondria induces production of reactive oxygen species (ROS), which damages DNA, RNA, and proteins, and thus becomes the leading cause of both aging and cancer. By triggering calcium homeostasis, Mara-Gen can reduce the production of ROS, restore cellular signaling, repair mitochondria dysfunction, build a strong immune system, and promote stem cell activities.

Pathogens and cancer cells thrive in an acidic environment due to the lack of oxygen in the body. When the blood pH is restored to ideal 7.4, our immune system is optimized to detect and destroy pathogens and reduce inflammation. Mara-Gen helps our body to neutralize the acid and restore the ideal body pH.



A GOLF BALL SIZED TUMOR - GONE!

I am a US Air Force pilot and one day was diagnosed with a brain tumor the size of a golf ball. I was devastated. However, when I heard about AIC therapy, I participated in it for three months before I got another brain scan at the Washington Bethesda Medical Center. The doctor asked me incredulously, "How come I do not see the tumor?"

Mara-Gen is a miracle supplement!

Merry - Female, 30.

MEGA-GEN

FORMULA FOR OSTEOPOROSIS AND BONE HEALTH

Osteoporosis and hormonal imbalances start affecting many people in their middle ages. Utilizing revolutionary AIC technology that delivers ionic calcium directly to our blood plasma, Mega-Gen is formulated to trigger our body's natural responses to balance our hormones and initiate the cascade of repairing processes with manifold health benefits including maintaining healthier bones to prevent osteoporosis.

The most noticeable result is strengthening of our bones by stabilizing calcium homeostasis. It is formulated to be a maintenance product for healthy individuals who want to fight aging-related bone loss and subsequent disruption of calcium homeostasis, which ultimately leads to hosts of degenerative diseases. Mega-Gen is truly a champion in maintaining optimum health, considering its manifold health effect.

Even as an antioxidant, Mega-Gen is unmatched in its potency and effectiveness compared to other products at a matching price range. Mega-Gen does what Mara-Gen can at maintenance strength. Benefits of Mega-Gen are too many to list.



IMPROVEMENTS WITH SLEEP, KNEE PAIN & DIABETES

I decided to try Mega-Gen to lower my blood sugar level. From the very beginning of taking it, my level dropped by 20 mg/dL and leveled out. I was excited and wanted more benefits, but I also knew I had to exercise and stop drinking. Then I noticed that I was sleeping much deeper and woke up more refreshed without waking up in the middle of the night. Also, I noticed that my left knee, which bothered me so that I had to hold handrails and take a few steps between rests, suddenly felt much better. The benefits I experienced from AIC therapy surpassed anything I tried at any cost! And I tried it for one month only!

S. L. – Female, 40, Indonesia

CARTI-GEN

FORMULA FOR ARTHRITIS AND JOINT HEALTH

Life takes a toll on our bones and joints. With age comes painful joints caused by wear and tear of cartilage and bones, and unfortunately, modern medical science only provides solutions that may ease pains and inflammations with many side effects. Also, because there are no blood vessels delivering nourishment to joints readily, the natural healing process of joints is too slow for most patients to overcome daily damage.

Ionic calcium moves freely in and out of blood vessels to reach the joint matrix and initiate cartilage repair by stimulating stem cells in the cartilage, namely chondrocytes. Also, anti-inflammatory and immune-normalizing effects of ionic calcium offer a scientifically superior solution for easing joint disorder pains. Formulated for cartilage repair, Carti-Gen helps more ionic calcium to permeate deeper into the joints to help rebuild damaged cartilage (stem cell action) and by removing painful mineral deposits. (calcium homeostasis)

For faster results, Mara-Gen can be taken together with Carti-Gen.



10 YEARS OF ARTHRITIS PAIN NOW SUBSIDING

I was hit with rheumatoid arthritis about 10 years ago. To escape stiffness and pain, I tried everything. My fingers were bent and knee pain was unbearable. My toes also hurt so much that I could not walk properly. Numerous doctor visits gave me different diagnosis such as virus, genetics, etc., which did not help me at all. On February 8, 2017, my friend introduced me to AIC therapy. After taking it for two weeks, the pain subsided dramatically, and now I can walk normally with my toe touching the ground! Two weeks of Carti-Gen is taking care of my 10-year-old problem!

Jung C. – Female, 66, Canada

ALZI-GEN

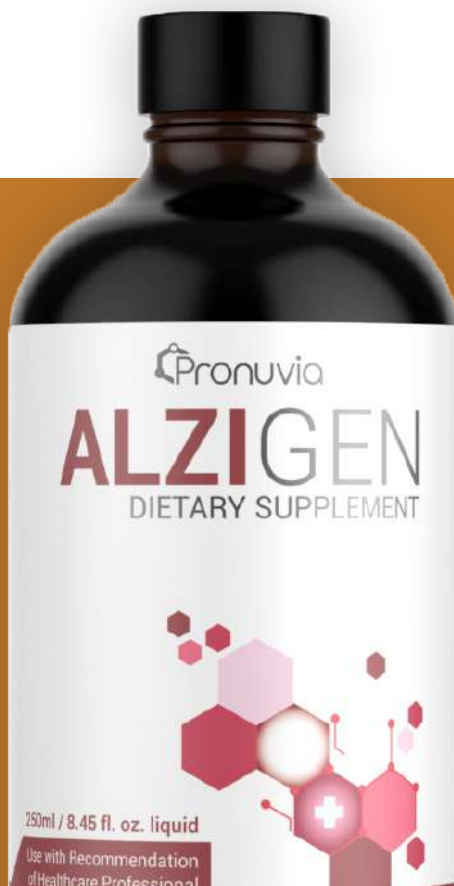
FORMULA FOR BRAIN HEALTH



The global cost of Alzheimer's disease (AD) and dementia is estimated to be \$605 billion. However, there are no drug treatments available that can provide a cure for Alzheimer's disease yet. Many health professionals label AD as type 3 diabetes, and others suggest age, genetics, and mineral imbalances as the possible cause of AD.

New research has linked the onset of Alzheimer's disease to disturbance of calcium homeostasis in the brain cells, which alters calcium signaling pathways that account for at least three major and interrelated toxic pathways: oxidative stress, mitochondrial dysfunction, and neuroinflammation in neurodegenerative diseases.

The aging-associated progressive loss of bone mass is largely to blame for this disturbance of calcium homeostasis, which in turn leads to neurodegenerative diseases. According to recent scientific research, senior women who have low bone density are more than twice likely to develop Alzheimer's disease. It indicates that AD is closely linked to osteoporosis. Triggering osteoblast to clear cellular calcification helps restore calcium homeostasis in cells and has thus shown promising results for AD patients.



Dear Dr. Lee,

Here is an update of the patient with Parkinson's disease. He is 75 years old, and his Parkinson's was not hereditary but caused by severe stress from his work. He started out with AIC therapy since October of 2017 without many expectations. However, after others commented that he seems to be doing much better, he was encouraged to take it faithfully through November, two doses a day. He said his tremors almost disappeared! After the onset of the disease, his life quality was miserable but now lives a new life. He came to my church to share the good news with me!

H. Shim

DENTI-GEN



FORMULA FOR IMPROVING DENTAL HEALTH

AIC therapy raises plasma calcium ion level and stimulates the release of TH and PTH that balances calcium homeostasis and increases the bone turnover rate to build strong bones. This process helps to rebuild alveolar bones of maxillary and mandible, hence increasing the rate of successful implants or periodontal treatments.

When working in conjunction with bone grafts, Denti-Gen accelerates the rebuilding of extraction sockets, promotes the integration of grafted bones, and assures better osseointegration of implant materials. Complementing bone grafts with Denti-Gen not only speeds up the procedure but also increases the success rate, even treating severe cases possible for dentists.

Human periodontal ligament stem cells (hPDLs) in periodontal ligaments differentiate into root cementum or alveolar bones and are known to be optimally active when the ratio of calcium and phosphorus ions is at 2:1. Denti-Gen helps the stimulation of hPDLs by bringing calcium to phosphorous ratio to this ideal level. Denti-Gen is also prescribed for strengthening loose teeth and for treating diseased gums.

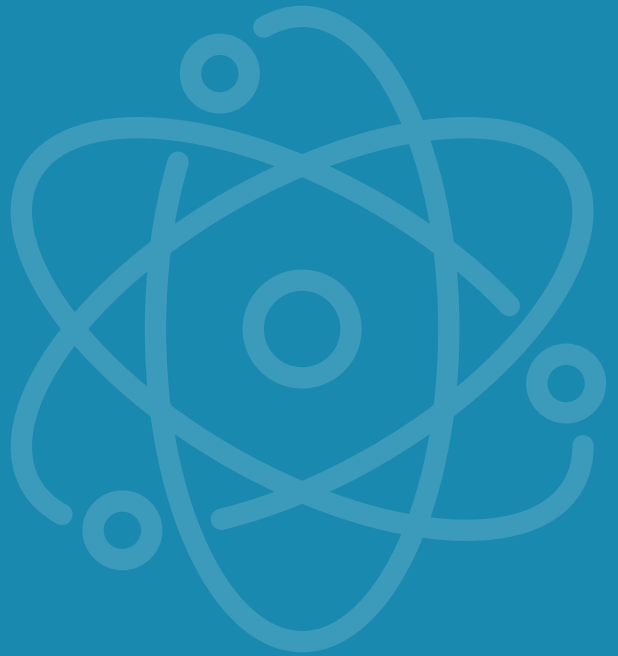


“ NO BONE GRAFT NEEDED FOR IMPLANT

My dentist extracted my molar tooth with a cavity six months ago. He recommended that I do an implant after a bone graft. I then remembered hearing about Denti-Gen and told my dentist that I would come back after increasing my jaw bone density. He assured me that there is simply no way to increase jaw bone density. After taking Denti-Gen for a few months faithfully, I revisited the dentist. He could not believe what he saw in X rays! He confirmed that the implant is now doable due to increased jaw bone density. Wow!

Soonok C – Female, 47

UPCOMING PRODUCTS



MARA-GEN INTENSIVE

We have always been trying to push the maximum ionic calcium content in Mara-Gen. We are about to have another breakthrough, pushing the concentration high enough to multiply the effects of already powerful Mara-Gen. Mara-Gen-Intensive is coming, and will increase the efficacy of AIC for elderly and for those fighting cancer.

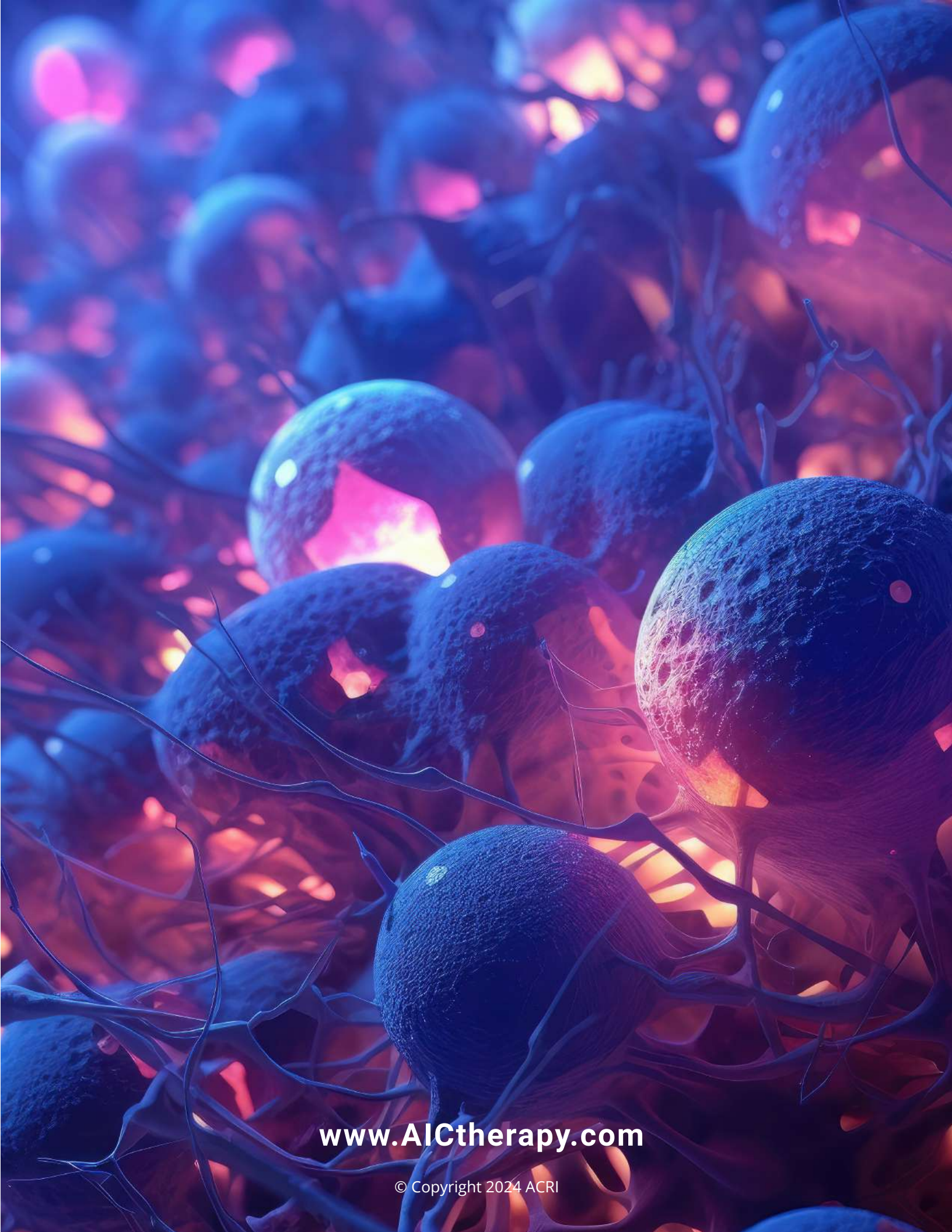
AIC-FA (Fatty Acid)

Cancer is very smart and can adapt or change its metabolism to use different sources of energy if glucose is not readily available. Working similarly as AIC-SG, AIC-FA is formulated to target cells that rely on fatty acids for survival. Combining AIC-FA with AIC and AIC-SG, we create a net that can trap pathogenic cells even tighter.

BIOMENULIN

Comes in powder form that can be mixed with favorite drinks, Biomenulin can reach colon to strengthen gut health and fight pathologic cells. Biomenulin is designed to release ionic calcium in colon and is a powerful complementary product when targeting Crohn's Disease or colon cancer.





www.AICtherapy.com

© Copyright 2024 ACRI